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Tactical Doctrine

MEDICAL GLOBAL REACH LAYDOWN TEAM (FFGRL)

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The Air Force Tactics, Techniques and Procedures (AFTTP) 3-42 series of publications is the primary reference for medical combat support capabilities. AFTTP 3-42.24 provides Tactics, Techniques and Procedures (TTP) for Medical Global Reach Laydown Team (FFGRL) personnel and medical equipment package (FFGR1) Unit Type Codes (UTCs) for execution of missions as outlined by Department of Defense Directive (DoDD) 3000.10, *Contingency Basing Outside the United States*, and Air Force Instruction (AFI) 10-404, *Base Support and Expeditionary (BAS&E) Site Planning*. The doctrine in this document is authoritative but not directive. This publication applies to all Department of the Air Force (DAF) civilian employees and uniformed members of the Regular Air Force, the Air Force Reserve (AFR), and the Air National Guard (ANG). This publication does not apply to the United States Space Force. For the purpose of this TTP, ANG is included in the term Major Command (MAJCOM). Ensure all records generated as a result of processes prescribed in this publication adhere to AFI 33-322, *Records Management and Information Governance Program*, and are disposed In Accordance With (IAW) the Air Force Records Disposition Schedule, which is located in the Air Force Records Information Management System. Refer recommended changes and questions about this publication to the Office of Primary Responsibility (OPR) using DAF 847, *Recommendation for Change of Product*, routed from the field through the appropriate functional chain of command.

SUMMARY OF CHANGES

This document is substantially revised and must be reviewed in its entirety.

Chapter 1 – INTRODUCTION
1.1 Overview....4
1.2 Capabilities....4
1.3 Supported Missions....5
1.4 Team Composition....5
1.5 Relation to Follow-On Medical Elements....5

Chapter 2 – ROLES AND RESPONSIBILITIES
2.1 Overview....6
2.2 618th Air Operations Center....6
2.3 Headquarters Air Mobility Command Surgeon....6
2.4 Pilot Unit....6
2.5 Medical Group Commander....6
2.6 Medical Group Medical Readiness....7
2.7 Medical Group Medical Logistics, War Reserve Materiel....7
2.8 Contingency Response Element Commander....7
2.9 Medical Global Reach Laydown Team Members....7
2.10 Tailored Team....9

Chapter 3 – COMMAND AND CONTROL AND COMMUNICATIONS SYSTEM SUPPORT
3.1 Overview....10
3.2 Command and Control....10
3.3 Operational Command Relationships....10
3.4 Operational Communications....10
3.5 Computers/Information Management/Technology....10

Chapter 4 – PRE-DEPLOYMENT/PLANNING
4.1 Overview....11
4.2 Operational Readiness....11
4.3 Medical Intelligence....12
4.4 Bed-down Considerations....14
4.5 Pre-Deployment Documentation and Reporting....14

Chapter 5 – OPERATIONS/DEPLOYMENT
5.1 Overview....16
5.2 Team Member Deployment Duties....16
5.3 Site/Airfield Survey....21
5.4 Emergency Response....21
5.5 Medical Chemical, Biological, Radiological, Nuclear Operations....21
5.6 Reporting....21
5.7 Security....22

Chapter 6 – REDEPLOYMENT AND POST DEPLOYMENT
6.1 Overview 23
6.2 Redeployment Notification 23
6.3 Pre-Redeployment Data Transition 23
6.4 Post Deployment Reporting 23

Chapter 7 – LOGISTICS
7.1 Overview 25
7.2 Medical Global Reach Laydown Equipment 25
7.3 Contingency Response Element Support 25
7.4 Transportation 25
7.5 Re-supply, Reconstitution/Sustainment 25
7.6 Vehicles 25
7.7 Patient Support 25

Chapter 8 – INFORMATION MANAGEMENT
8.1 Overview 26
8.2 Information Assurance Policy 26
8.3 Communication Systems 26
8.4 Software Applications 26
8.5 Data Transition 28
8.6 Deployment Medical Records 28
8.7 Help Desk Support 29

Chapter 9 – EDUCATION AND TRAINING
9.1 Overview 30
9.2 Training Responsibilities 30
9.3 Formal Training 30
9.4 Equivalency Credit 31
9.5 Unit Level Training 31
9.6 Exercises 32

ATTACHMENTS
1. GLOSSARY OF REFERENCES AND SUPPORTING INFORMATION 33
2. FLIGHT MEDICINE RECOMMENDED REFERENCES 43
3. BIOENVIRONMENTAL ENGINEERING RECOMMENDED REFERENCES 45
4. PUBLIC HEALTH RECOMMENDED REFERENCES 47
5. EXAMPLE SITUATION REPORT 49
6. OPERATING CAPABILITY CONDITIONS 50
7. CONTINGENCY RESPONSE FORCE PACKING LIST AND EQUIPMENT 52
8. EXAMPLE SITE SURVEY CHECKLIST 54
9. AFTER ACTION REPORT TEMPLATE 68
10. CHECKLISTS 71

Chapter 1

INTRODUCTION

1.1. Overview. This document outlines the functions and responsibilities of the FFGRL across all phases of operation, including pre-deployment/planning, deployment, re-supply/sustainment, redeployment, and post deployment activities. FFGRL is a personnel UTC that must be paired with the FFGR1 medical equipment package UTC to effectively support contingency operations. Together, FFGRL and FFGR1 provide limited medical care for injuries and illnesses, as well as preventive medical services, in support of Contingency Response Elements (CREs). Operationally, FFGRL falls under Operational Control (OPCON) of the commander, AMC (COMAMC) and typically is not allocated to a theater unless specifically required. Exceptions apply to medical personnel assigned to CREs that are not aligned under COMAMC; in such cases, their OPCON will remain within the respective theater to which they are assigned.

1.2. Capabilities. FFGRL capabilities are specifically designed to mitigate the impact of Disease and Non-Battle Injuries (DNBI) on mission success during contingency operations, while providing limited medical support. The team is deployed to assess health risks associated with Occupational and Environmental Health (OEH) hazards and offers the following capabilities to support CRE operations:

1.2.1. Provides medical expertise to ensure the proper layout and placement of facilities.

1.2.2. Delivers limited lifesaving trauma care (Tactical Combat Casualty Care) and Role 1 medical care to assess, treat, and return personnel to duty following DNBI.

1.2.3. Assesses health and safety standards for local billeting, public facilities, and local food and water sources.

1.2.4. Evaluates local medical capabilities and recommends locations for medical facilities.

1.2.5. Leverages medical intelligence to deliver medical support, conduct disease and injury surveillance, and develop emergency planning and response strategies.

1.2.6. Assesses risks associated with Toxic Industrial Chemicals and Toxic Industrial Materials (TIC/TIM).

1.2.7. Conducts vector and pest risk assessments, evaluates the risk of vector-borne diseases, and monitors compliance and effectiveness of vector and pest management programs. Recommends prevention and control measures while providing education and training on vector-borne disease prevention and control.

1.2.8. Anticipates chemical, biological, radiological, and nuclear (CBRN) health threats by leveraging medical intelligence and prior site planning. Identifies potential health hazards and implements appropriate controls based on relevant intelligence.

1.3. Supported Missions.

1.3.1. Contingency Response Element (CRE). FFGRL/FFGR1 teams are assigned to CREs to deliver medical support during the rapid opening of contingency airfields. The primary objective of the group is to ensure significant order, foresight, speed, and safety during the critical initial days of a contingency operation. Refer to AFI 10-202, *Contingency Response Forces* for more information on CRE organization, mission sets, deployable capabilities, and functions. FFGRL provides preventive medicine support in the following CRE mission sets:

1.3.1.1. CRE Advanced Echelon (ADVON) team's initial laydown activities.

1.3.1.2. Joint Port Opening Capability.

1.3.1.3. Air Base Opening.

1.3.1.4. Support to the Global Air Mobility Support System.

1.4. Team Composition. The team is composed of four personnel equipped with necessary medical resources. It is specifically designed for light mobility, enabling timely preventive medicine support to meet the operational demands of the CRE. In deployed settings, FFGRLs are fully integrated into the overall team composition to ensure seamless functionality. The FFGRL team chief, serving as the senior medical officer, reports directly to the CRE commander to maintain effective communication and leadership. **Note:** Medical care can only be provided if the team includes a Flight Surgeon (048R3) and/or an Independent Duty Medical Technician (4N071C) as part of the package.

Position Title	Air Force Specialty Code (AFSC)	Grade	Quantity
Residency Trained Flight Surgeon (may substitute 048G3 with HQ AMC/SGP approval only)	048R3 *or* 048A3	O-4	1
Public Health Officer *or* 7-Level Public Health Technician	043H3 *or* 4E071	O-3 E-6/E-7	1
Bioenvironmental Engineering Officer *or* 7-Level Bioenvironmental Engineering Technician	043E3 *or* 4B071	O-3 E-6/E-7	1
Aerospace Medical Service Journeyman/ Independent Duty Medical Technician (IDMT)	4N071C	E-6/E-7	1

1.5. Relation to Follow-On Medical Elements. As part of the CRE, FFGRL is typically the first medical asset to arrive at a deployed location. The team is responsible for conducting initial preventive medicine actions to address immediate health risks and operational needs. These actions should be seamlessly transferred to follow-on forces, such as the Preventive and Aerospace Medicine (PAM) Team or other preventive medicine assets, upon their arrival to ensure continuity of care and mission support.

Chapter 2

ROLES AND RESPONSIBILITIES

2.1. Overview. Force health protection and casualty prevention are influenced by numerous interconnected elements. This section outlines these elements and highlights the linkages between Line of the Air Force operations and Health Service Support assets. Commanders at all levels bear the responsibility for safeguarding Air Force personnel. Effective Command and Control (C2) structures are essential to enable commanders to swiftly and efficiently address passive defense concerns and respond to force health protection threats. FFGRL personnel play a critical role in identifying potential health hazards, developing actionable courses of action, and advising commanders on associated risks and recommended controls to mitigate those risks.

2.2. 618th Air Operations Center (618 AOC). The 618 AOC is responsible for controlling operational missions that support global mobility operations across all Combatant Commands (CCMDs). Located at Scott Air Force Base, IL, the 618 AOC operates 24/7, 365 days a year, transforming mission requirements into effective operations worldwide. As AMC's center of expertise, the 618 AOC specializes in planning, tasking, executing, and assessing mobility missions, serving as an invaluable resource for maintaining America's global reach. Its directorates oversee a wide range of critical functions, including current operations, C2, Contingency Response (CR), Aeromedical Evacuation (AE), aerial port operations, en-route maintenance, mission management, flight planning, deployment, diplomatic clearances, communications, weather, and intelligence for strategic and theater airlift, as well as air refueling aircraft. The centralized management and tasking for similar teams are handled by the 603rd Air Operations Center for Headquarters United States Air Forces in Europe – Air Forces Africa (HQ USAFE-AFAFRICA) and the 613th Air Operations Center for Headquarters Pacific Air Forces (HQ PACAF).

2.3. Headquarters Air Mobility Command, Command Surgeon (HQ AMC/SG). As the UTC MEFPAK Responsible Agent (MRA), HQ AMC/SG is tasked with designating and sourcing medical units to support FFGRL teams with appropriately deployable personnel and FFGR1 medical equipment packages. HQ AMC/SG is also responsible for overseeing UTC coordination and validation processes, ensuring the timely completion and verification of Logistics Detail (LOGDET) information for updated Allowance Standards (AS). During deployment, HQ AMC/SG will receive medical reports from both the FFGRL and the CRE. For UTCs assigned to locations outside the Continental United States, medical reports will be sent to the respective Commands' SG offices.

2.4. Pilot Unit. FFGRL/FFGR1 pilot unit is the 87th Medical Group, located at Joint Base McGuire-Dix-Lakehurst (JBMDL), New Jersey. This unit is responsible for developing and maintaining the FFGR1 AS and LOGDET.

2.5. Medical Group Commander (MDG CC). MDG CC with assigned Medical Resource Letter (MRL) FFGRL personnel are responsible for ensuring that team members are properly appointed and trained, as well as maintaining the readiness of the FFGR1 AS. Additionally, the MDG CC will provide necessary support to facilitate successful pre-deployment planning,

deployment, redeployment, and after-action reporting.

2.6. Medical Group Medical Readiness. Refer to roles and responsibilities as defined in Department of the Air Force Instruction (DAFI) 41-106, *Medical Readiness Program*.

2.7. Medical Group Medical Logistics, War Reserve Materiel (WRM). IAW DAFI 41-230, *Medical Logistics Support*, the MDG CC will appoint a medical WRM Project Officer. This role is typically assigned to the Accountable Base Medical Supply Officer (ABMSO) but may also be filled by a military member at the grade of E7 or above, or a federal government employee at the General Schedule (GS)-09/Wage Grade (WG) equivalent or higher, working within the Medical Logistics (MEDLOG) Flight. WRM Project Officers are responsible for ensuring that all authorized contingency medical materiel assemblages are properly established, inventoried, reviewed, validated, packaged, stored, and reported in the Defense Readiness Reporting System-Strategic (DRRS-S).

2.8. Contingency Response Element (CRE) Commander. The CRE commander, hereafter referred to as the deployed commander, is responsible for ensuring the force health protection of deployed personnel. The deployed commander relies on the advice and expertise of FFGRL functional area experts to ensure that all identified health issues and associated controls are documented in the written medical site assessment and medical laydown plan. Additionally, the deployed commander must integrate the FFGRL team and any additional Expeditionary Combat Support (ECS) assets into their overall deployment plans to ensure comprehensive mission readiness and support.

2.9. Medical Global Reach Laydown Team (FFGRL) Members. FFGRL members deliver preventive medical support to help the deployed commander gain a thorough understanding of the medical aspects of the operational environment. They provide expert advice on force health protection measures to mitigate potential hazards and ensure the safety and well-being of deployed personnel.

2.9.1. Force Readiness. The MDG supporting the deploying unit plays a critical role in enabling the FFGRL mission by preparing personnel for contingency operations. All FFGRL members must maintain a continual state of readiness to support taskings with minimal notice. Therefore, it is essential for all members to actively participate in the military's fitness program to ensure a fit and ready force. The Medical Readiness Office is responsible for verifying that personnel assigned to the FFGRL UTC meet the CR Officer or Enlisted Personnel Processing Codes (PPCs) prior to their assignment to the UTC. Additionally, FFGRL members must maintain an Individual Medical Readiness (IMR) status of "green" and be included in the same immunization group as the CR units within the Aeromedical Services Information Management System (ASIMS) to ensure worldwide mobility.

2.9.2. Team Chief. The senior medical officer of the FFGRL serves as the team chief. In the absence of the senior medical officer, the individual with the next highest rank will assume leadership responsibilities. The team chief is responsible for overseeing all casualty prevention activities and ensuring that health risks and recommended controls are

effectively communicated to the deployed commander for inclusion in the written medical site assessment and medical laydown plan. Additionally, the team chief is tasked with conveying the operational needs of the FFGRL to the deployed commander and overseeing the inventory of the FFGR1 upon arrival and prior to departure.

2.9.3. Flight Surgeon (FS). The FS serves as the chief medical advisor to the deployed commander, providing expert guidance on all medical issues. The FS is responsible for implementing and directing a coordinated base medical program until definitive sustainment medical support, such as Expeditionary Medical Support (EMEDS), arrives. The FS will establish and coordinate Standard Operating Procedures (SOPs) for critical medical components, including medical evacuation, In-Flight Emergency (IFE) response, aircraft mishap response, and mass casualty response. In addition, the FS provides medical oversight for flying and special operations, prescribes prophylaxis and pre-treatments, evaluates health screenings, coordinates and delivers limited medical care, and addresses human factor issues such as recurring training, crew resource management, and counter-fatigue measures. The FS also serves as the preceptor for IDMTs, ensuring their professional development and operational readiness.

2.9.4. Public Health (PH). PH plays a vital role in safeguarding the health and readiness of deployed personnel by focusing on disease prevention, mitigating environmental hazards, ensuring food safety, and maintaining field sanitation standards. PH develops medical intelligence assessments and collaborates with Bioenvironmental Engineering (BE) to create comprehensive Health Risk Assessments (HRAs). Food and water risk assessments (FWRAs) may be conducted by PH officers who have completed the U.S. Army FWRA Course. PH is responsible for overseeing food safety and security, communicable disease control, occupational health education, site selection, insect and vector monitoring and control recommendations, immunization policies, and enhanced surveillance activities. Additionally, PH monitors disease trends, tracks DNBI rates, and conducts preliminary Food Vulnerability Assessments (FVAs) to identify potential overt or covert use of biological warfare agents as an early warning measure.

2.9.5. Bioenvironmental Engineering (BE). The BE member initiates the Occupational and Environmental Health Site Assessment (OEHSA) process by conducting a Preliminary Hazard Assessment (PLHA) and establishing a baseline evaluation of OEH conditions. The BE member utilizes relevant medical intelligence to identify potential OEH and CBRN health hazards and appropriate controls to mitigate risks.

2.9.6. Independent Duty Medical Technician (IDMT). The IDMT serves as a medical provider extender, augmenting the capabilities of the deployed FS. The IDMT provides limited medical, dental, and emergency treatment to the deployed population IAW AFI 44-103, *The Air Force Independent Duty Medical Technician Program*. In the deployed environment, the IDMT performs pharmacy, laboratory, immunizations, and medical logistics duties and is authorized to dispense controlled substances under the direction of the FS. The IDMT also possesses limited specialized training in BE and PH operations and will perform all duties within their scope of training until BE or PH personnel arrive on site. Once BE and PH personnel are available, the IDMT will collaborate with them to augment

their activities under the direction of the BE/PH team member. During initial site surveys, and with appropriate reach-back support from BE/PH teams, the IDMT can conduct limited environmental monitoring, water and food safety assessments, field sanitation, pest/vector surveillance, and provide education on heat/cold stress to deployed units.

2.10. Tailored Team. FFGRL/FFGR1 packages can be tailored based on the deployed location and mission requirements. During the planning phase, CR commanders should consult with their respective MAJCOM SG Offices to explore tailoring options. Examples include:

2.10.1. The mission requires a small CR force package and/or poses no significant public health, environmental, or industrial concerns.

2.10.2. BE must be included in the team if the mission is to an industrialized area where significant environmental or industrial health concerns may exist (e.g., Eastern Europe).

2.10.3. PH must be included in the team in areas where flora, fauna, or endemic diseases are of primary concern due to a high threat of vector-borne diseases (e.g., Africa, Southeast Asia).

Chapter 3

COMMAND AND CONTROL AND COMMUNICATIONS SYSTEM SUPPORT

3.1. Overview. Command, Control, Communication, and Computers (C4) assets must be identified during the planning process to ensure they are effectively deployed to support both tactical and strategic C2 mission requirements.

3.2. Command and Control (C2). The air component commander's AOC provides C2 of air operations to execute all requirements established by the JFC. Typically, the Air Mobility Division (AMD), or its theater equivalent, is responsible for centralized planning, tasking, scheduling, and coordination. The AOC executes C2 of assigned and attached air mobility forces operating within the Combatant Commander (CCDR) 's area of responsibility. Additionally, the AMD in coordination with the Director of Mobility Forces (DIRMOBFOR), the Joint Deployment Distribution Operations Center (JDDOC), and the 618 AOC integrates intertheater and intratheater air mobility operations to ensure the mission is accomplished efficiently and effectively.

3.3. Operational Command Relationships. Command Relationship (COMREL) is aligned with the deployed CRE. OPCON of the deployed CRE is retained by its respective CCDR and Air Component to which it is assigned, unless the Secretary of War reassigns OPCON. Since FFGRL/FFGR1 teams are not typically assigned to the supported CCMD, it is essential for them to maintain close coordination with the supported Commander, Air Force Forces (COMAFFOR) medical staff. This ensures a seamless transition to follow-on definitive medical support.

3.4. Operational Communications. Access to the Defense Switching Network (DSN), DSN modem, secure telephone, e-mail, internet, and a secure messaging center may be provided by the communications personnel at the deployed location. FFGRL members must deploy with both a Non-Classified Internet Protocol Router Network (NIPRNet) account and a Secret Internet Protocol Router Network (SIPRNet) account to effectively receive and transmit sensitive medical information.

3.5. Computers and Information Management/Technology. Computers are essential tools for accessing reference sources and a wide range of medical information, including disease and hazard surveillance. Deployed computers should be ruggedized and equipped with wireless capability to interface seamlessly with the CRE's communications packages. The hardware enables effective communication for reporting and information transfer, ensuring mission-critical data is shared efficiently. For detailed Information Management practices, refer to Chapter 8.

Chapter 4

PRE-DEPLOYMENT/PLANNING

4.1. Overview. FFGRL personnel are aligned with the Air Force Force Generation (AFFORGEN) model. During the PREPARE phase, efforts are focused on maintaining FFGRL member qualifications, ensuring the readiness of FFGR1 equipment, and planning for the employment of medical assets to support the CRE. FFGRL/FFGR1 UTCs are identified in a unit's MRL. While the associated medical equipment packages are stored at a corresponding host Air Force base, they are not necessarily co-located with FFGRL personnel.

4.2. Operational Readiness.

4.2.1. Deployment Readiness. Upon assignment to FFGRL, all members are required to obtain official passports, SIPRNet accounts, Department of Defense (DoD) Platform One Impact Level 4 (IL4) accounts and complete all necessary training requirements through their Medical Readiness Office.

4.2.2. Personnel assigned to FFGRL must be familiar with FFGR1 equipment, Mission Capability (MISCAP) statements, and operational employment procedures. Team members will be thoroughly indoctrinated with the specific requirements of their role. Upon notification of a potential FFGRL deployment, team members will conduct a comprehensive personnel and local equipment readiness review to identify any shortfalls. Additionally, transportation requirements for FFGR1 UTC equipment should be identified, and equipment must be prepared for movement. The team chief is responsible for ensuring that at least two team members are qualified in Pallet Build-Up and Hazardous Material Preparers procedures. This includes handling hazardous, protected, and classified cargo to serve as cargo increment monitors during deployment and redeployment, IAW DAFI 10-403, *Deployment Planning and Execution*.

4.2.2.1. Personnel Preparation. Personnel assigned to FFGRL must be fully trained and maintain a continuous state of readiness. Additionally, FFGRL members should prepare and maintain a ready kit that includes the publications outlined in the attachments of this AFTTP. Upon notification of deployment, the deploying FS or team leader must coordinate with their assigned MDG Medical Readiness Office and leadership to ensure compliance with deployment timeline requirements.

4.2.2.2. FFGRL team chief is responsible for ensuring that all deploying personnel meet the tasking requirements specified by the supported command. Additionally, the team chief must ensure that hot, cold, wet, or other weather gear necessary to support the CREs is provided IAW AFTTP 3-4.7, *Contingency Response*.

4.2.3. The FFGRL pilot unit should establish practices to ensure familiarity with equipment UTCs and pack-out guides and report any identified shortfalls to their MRA at least once per calendar year. Teams will deploy with a detailed listing of the assemblage for pack-out and re-supply actions. FFGRL personnel may be required to complete their own Shipment

Declaration of Dangerous Goods forms for hazardous items included in their deploying equipment. These forms must be presented to the appropriate agencies IAW the base Installation Deployment Plan, with copies provided to the deploying commander.

4.2.3.1. Hazardous cargo must comply with Defense Transportation Regulation (DTR) 4500.9-R Part III, *Mobility*, Air Force Manual (AFMAN) 24-604, *Preparing Hazardous Materials for Military Air Shipments*, and host nation requirements. Personnel must adhere to the Department of Transportation requirements outlined in Title 49 Code of Federal Regulations Part 172 (49 CFR §172), *Hazardous Materials Table, Special Provisions, Hazardous Materials Communications, Emergency Response Information, Training Requirements, and Security Plans*.

4.2.3.2. If classified material is present, it must be packaged, marked, safeguarded, and transported IAW DAFI 24-602V2, *Cargo Movement*. Units are responsible for assigning couriers who are appropriately cleared and trained to handle and accompany classified material during transport.

4.3. Medical Intelligence. During the pre-deployment phase, PH should obtain a medical intelligence assessment to support pre-deployment briefings and to include in the medical site assessment and medical laydown plan. Key sources for medical intelligence include HQ AMC/PACAF/USAFE Aerospace Medicine Division (SGP) and Operational Medical Readiness Division (SGX) offices, base and inter/intra-theater intelligence, the Air Force Office of Special Investigations (AFOSI), the National Center for Medical Intelligence (NCMI) via NIPRNet e-mail (~DIANCMIOPS@dodiis.mil) and SIPRNet e-mail (ncmi_ops@dia.smil.mil), airfield survey reports, previous After Action Reports (AARs), the Joint Lessons Learned Information System (JLLIS), other published lessons learned sites, the Department of State, and the in-country embassy or consulate. Medical intelligence gathering should consider the following:

4.3.1. Infectious Disease Prevention and Risk Assessment. PH gathers information on local area disease hazards/vectors to determine required immunizations and chemoprophylaxis as outlined in CCMD theater medical entry requirements and TRAVAX.

4.3.2. Chemical, Biological, Radiological and Nuclear (CBRN) Threat Assessment. PH and BE should collaborate to assess host nation CBRN capabilities, as well as potential threats at the deployed location. This information can be obtained from the base intelligence office, the AFOSI, and the Defense Intelligence Agency website via SIPRNet. (https://www.dia.smil.mil).

4.3.3. Toxic Industrial Chemical/Toxic Industrial Material (TIC/TIM) Assessment/Industrial Health Hazards. BE gathers information on the presence of TICs and TIMs in the area of operations. Local activities such as manufacturing, mining, or agriculture can pose risks that include DNBIs from incidental or accidental releases, as well as battle injuries resulting from enemy actions or threats. Efforts should be made prior to site selection to minimize the risk of exposure to industrial chemicals and materials by optimizing the location selection process. In addition to the sources used for assessing risks from the ambient environment, personnel may research site surveys, historical or National Imagery and

Mapping Agency images, and collaborate with the AFOSI to access additional intelligence sources.

4.3.4. Environmental Health Hazards. BE should gather intelligence on human health hazards associated with environmental conditions at the deployed location. Key areas to address include the following:

4.3.4.1. Previous OEHSAs or Base Camp Assessments available within the Defense Occupational and Environmental Health Readiness System (DOEHRS).

4.3.4.2. Pre-Deployment Site Surveys, DOEHRS Environmental Health (DOEHRS-EH) business area, and Environmental Health Risk Assessments from NCMI.

4.3.4.3. Local climate and weather data from the National Climatic Data Center (NCDC) https://www.ncei.noaa.gov/cdo-web/ for thermal stress assessments and countermeasures, as well as hydrological and topographical conditions for site selection.

4.3.4.4. Ambient air quality. Potential resources include the Environmental Protection Agency (EPA) AirNow https://www.airnow.gov/?city=false&country=false and the Real-time Air Quality Index Visual Map https://aqicn.org/map/world.

4.3.4.5. Raw water source selection, including surface and groundwater. Useful resources include the Food and Agriculture Organization of the United Nations AQUASTAT database https://www.fao.org/aquastat/en/, and the National Integrated Drought Information System https://www.drought.gov/international/.

4.3.4.6. Municipal water sources and associated hazards, including organic and inorganic chemicals, bacteriological treatment methods, and infrastructure. Water quality data may be available through The National Water Quality Monitoring Council https://www.waterqualitydata.us/.

4.3.4.7. Bottled water sources approved by the Defense Health Agency-Public Health and/or contracted local vendors. Approved bottled water sources can also be verified in the Veterinary Service Information Management System (VSIMS) database. BE and PH must ensure that contracted local vendors are approved sources.

4.3.5. Prior to departing home station, review the MISCAP statements, Manpower Force documents, and equipment allowance standards of the CRE and other potential expeditionary support capabilities that may deploy with the team. Based on the equipment outlined in supporting UTCs, hazards can be identified and assessed. The assessment may indicate the need to establish engineering or administrative controls to limit exposure and/or identify situations where Personal Protective Equipment (PPE) may be required or recommended. If administrative controls are necessary, they should be provided to the supervisor or commander for inclusion in training and operational guidance. If PPE is required, it must be included in the Operation Order (OPORD) and reporting instructions to

ensure proper implementation during deployment.

4.3.6. BE and PH are responsible for assessing the availability, safety, and vulnerability of potential food and water sources. Bottled water will serve as the primary source of drinking water until the safety of local water resources can be verified. Initially, food will be provided in the form of Meals, Ready to Eat (MREs). **Note:** A FWRA is required if MREs and/or approved sources are unavailable. Once completed, the FWRA must be documented in VSIMS. Environmental Health Risk Assessment reports from the NCMI are valuable pre-deployment resources for identifying vulnerabilities. Additionally, the Worldwide Directory of Sanitarily Approved Food Establishments for Armed Forces Procurement will be consulted to identify local sources of approved food and water.

4.4. Bed-down Considerations. Pre-deployment mission planning must account for any location being considered as a bed-down area. Bed-down considerations should include factors such as local climate, seasonal flood plains, prevailing winds, and the history of land usage, as these elements can impact support facilities such as dining halls, latrines, and sleeping accommodations. Planning must be conducted IAW AFI 10-202, *Contingency Response Forces*. Any pre-deployment site surveys must include qualified preventive health personnel, such as those in the 48 series, 43H, 43E, 4E0X1, 4B0X1, and 4N0X1C career fields, to ensure thorough assessment and planning. These assessments should be conducted as part of the BAS&E process or during a CR ADVON site visit.

4.5. Pre-Deployment Documentation and Reporting.

4.5.1. All pre-deployment data must be submitted to the deployment theater HQ SGP. For program updates and additional information, visit the Joint Operational Medicine Information Systems (JOMIS) platform. For further support or inquiries, contact JOMIS through their Communications mailbox at: dha.ncr.peo-ipo.mbx.jomis-communications@health.mil.

4.5.2. Medical Site Assessment and Medical Laydown Plan. IAW AFI 10-202, *Contingency Response Forces*, FFGRL must initiate a comprehensive medical site assessment and medical laydown plan. This plan should include medical intelligence, vulnerability assessments, an evaluation of potential health risks, and recommended controls to mitigate those risks. The plan must be provided to the deployed commander and the respective Command's SGP office prior to deployment for integration into the overall force protection plan. Additionally, the plan must be continuously updated at the deployed location as new information becomes available.

4.5.3. Pre-Deployment Briefing. The local PH office is responsible for organizing a medical intelligence pre-deployment briefing. This briefing must be presented to the deployed commander and all deploying personnel at the home station prior to departure.

4.5.4. Medical Report for Emergencies, Disasters and Contingencies (MEDRED-C). IAW DAFI 41-106, *Medical Readiness Program*, the MEDRED-C is used to provide information on a unit's operational status during deployed (Part A) and in-garrison (Part B) contingency

operations, as well as during unusual occurrences such as natural disasters, prolonged power outages, civil unrest, CBRN events, or other emergencies. MEDRED-Cs are submitted through the Air Force Input Tool (AF-IT) within DRRS-S. Air Reserve Component (ARC) units are not required to submit MEDRED-Cs.

Chapter 5

OPERATIONS/DEPLOYMENT

5.1. Overview. This section outlines the duties of each FFGRL member during deployment, along with the associated personnel reporting requirements. Adjustments and reexamination of duties are necessary if all team members are not deployed or if the team chief is someone other than the FS. The FFGRL team can reach initial operational capability (IOC) within 24 hours of arrival and reach full operational capability (FOC) within 48 hours of arrival. See Attachment 6 for more information on IOC and FOC parameters.

5.2. Team Member Deployment Duties.

5.2.1. Flight Surgeon (FS). The FS is primarily responsible for administering aerospace, occupational, and preventive medicine programs. The FS establishes medical Standards of Practice and performs the following deployment operations:

5.2.1.1. Arranges IFE coverage and briefs deployed commander on IFE coverage and capabilities.

5.2.1.2. Establishes contact with the AE system and briefs the deployed commander on access procedures.

5.2.1.3. Initiates first contact with local health care authorities (e.g., Joint, coalition) and host nation representatives to arrange cross-coverage, evacuation, and referral procedures.

5.2.1.4. Assesses local medical capabilities and provides the results of the assessment to the deployed commander.

5.2.1.5. Develops and coordinates an aircraft mishap response and investigation plan.

5.2.1.6. In collaboration with PH, validates medical intelligence from the NCMI and TRAVAX, updates intelligence data, and identifies additional countermeasures.

5.2.1.7. Assesses the medical aspects of exposures, occupational illnesses, and injuries.

5.2.1.8. Works with BE and PH to inform the deployed commander of local diseases, occupational health risks, environmental health risks, CBRN health risks, and recommended controls.

5.2.1.9. Analyzes and compiles disease reports with PH and informs respective Command SGP offices and deployed commanders of any significant trends as needed.

5.2.1.10. Provides medical care to deployed personnel.

5.2.1.11. Monitors operations and briefs the deployed commander on critical issues impacting flying operations, including circadian rhythm disruptions, fatigue, heat/cold stress, and clinical and operational medical support affecting human performance optimization, sustainment, and safety-related concerns.

5.2.1.12. Precepts the IDMT to jointly provide basic essential medical care to deployed personnel. Additionally, the FS serves as a clinical liaison for members requiring treatment at another U.S. or coalition MTF. Acute ambulatory care can be provided for up to 200 personnel for a duration of up to 60 days. While the FFGRL has no patient holding capability, it can provide non-surgical stabilization for up to two trauma casualties simultaneously.

5.2.1.13. Monitors and reports on the status of medical capabilities to the deployed commander.

5.2.1.14. Arranges and coordinates transportation for emergency and/or mishap response, as well as BE and PH functions.

5.2.1.15. Serves as the OPR for the deployed medical site assessment, medical laydown plan, MEDRED-C, and Situation Report (SITREP).

5.2.1.16. Receives all Deployment Medical Records (DD Form 2766, *Total Force Health Readiness Flowsheet*) from the CRE Commander within 12 hours of arriving at the deployed location. Ensures that clinical documentation of occupational exposures and other medical concerns is preserved in the service members' medical records.

5.2.1.17. Ensures vehicle support requirements are communicated to the deployed commander prior to arrival for approval. At a minimum, one vehicle capable of transporting two litter patients and one vehicle to support BE and PH medical site assessment activities must be requested. Vehicle procurement may be arranged through local contracting efforts.

5.2.2. Bioenvironmental Engineering (BE). A Bioenvironmental Engineer (BEE) or BE technician provides specialized expertise in the OEHSA process, Periodic Occupational and Environmental Monitoring Summary (POEMS), OEH surveillance, medical CBRN defense, health risk assessments, documentation of potential exposures, and potable water analysis. Additionally, the BEE or BE technician performs the following deployment operations.

5.2.2.1. Occupational and Environmental Health Site Assessment (OEHSA) at Deployment Locations – PLHA. BE serves as the OPR for OEHSA products; however, all FFGRL members are responsible for initiating, implementing, and maintaining the PLHA throughout the deployment.

5.2.2.1.1. When conducting the PLHA, personnel should, to the extent practical, adhere to the guidance provided in AFTTP 3-2.82, *Occupational and Environmental Health Site Surveillance at Deployment Locations*.

5.2.2.1.2. Screening for health hazards should begin as soon as practical at the bed-down location. Information relevant to asset siting and base build-up must be reported to the deployed commander immediately. **Note:** The FFGRL commander must ensure that preventive medicine personnel are included in site selection for both the initial laydown and follow-on forces/assets.

5.2.2.2. Occupational and Environmental Health (OEH) Surveillance. The FFGR1 medical equipment package includes limited resources for conducting OEH surveillance. BE will, at a minimum, complete a preliminary health assessment for the site, which serves as the foundation for the OEHSA to be completed by follow-on preventive medicine teams upon their arrival. Most industrial activities are unlikely to have commenced during the early stages of the base opening construct.

5.2.2.2.1. Water distribution system analysis. BE and Civil Engineering (CE) personnel are generally responsible for drinking water surveillance at the deployed site and must perform appropriate potable water quality monitoring to minimize the risk of waterborne illness to deployed personnel.

5.2.2.2.1.1. Provides the CRE Commander with recommendations for approving water for consumption when water is not yet readily available. This includes testing and certifying potable water generating units and Reverse Osmosis Water Purification Units (ROWPUs).

5.2.2.2.1.2. The types and frequency of drinking water quality monitoring are determined by CCMD policy, AFMAN 48-138_IP, *Sanitary Control and Surveillance of Field Water Supplies*, and geographically specific threat conditions. At a minimum, BE must utilize the guidance provided in Table 4-2, "Short-term Potability Military Field Water Standards," from the above manual. If short-term potability testing has not yet been completed, water treated by reverse osmosis may still be provisionally released for drinking and other activities, provided that documented operational monitoring parameters confirm the proper functioning of the treatment equipment.

5.2.2.2.1.3. Conducts preliminary drinking water vulnerability assessments for the deployed site and advises the deployed commander on the associated risks.

5.2.2.2.2. Waste management. BE and PH shall collaborate with CE, when available, to provide guidance on appropriate PPE for handling various types of waste, including solid, biological, human, and hazardous waste. They will also ensure that storage and disposal sites are managed in a manner that does not pose health hazards to follow-on forces.

5.2.2.2.3. Hazardous Materials management. Anticipates, recognizes, evaluates, and documents hazardous conditions and materials associated with industrial, flightline, and contingency operations. Recommendations for exposure controls are based on the toxicity and usage of chemicals. When engineering and work practice controls are not installed or feasible, BE recommends appropriate PPE to mitigate exposures. BE also develops plans for the control and use of hazardous materials, works with deployed personnel to ensure proper management of hazardous materials, and advises the deployed commander on hazardous material incidents.

5.2.2.2.4. Thermal stress education. Coordinates with local weather services, if available, to ensure timely delivery of weather notifications. This enables deployed personnel to be properly educated on thermal stress risks and allows commanders to make informed decisions regarding work-rest cycles.

5.2.2.2.5. Radiation safety. Monitors work areas and personnel involved in the storage, use, and disposal of ionizing and non-ionizing radiation sources and generation devices. Educates deployed personnel on radiation exposure health risks and promotes adherence to the "As Low As Reasonably Achievable" (ALARA) principles.

5.2.2.3. Chemical, Biological, Radiological, Nuclear (CBRN). Provides expertise to the deployed commander on CBRN effects, health-based risk assessments, and operations in CBRN environments, in coordination with PH. Evaluates local industrial facilities that could impact base operations in the event of TIC/TIM releases.

5.2.3. Public Health. The PH officer or technician provides specialized expertise in food inspection and vulnerability assessments, food facility sanitation, medical intelligence, disease and injury surveillance, vector surveillance, field sanitation evaluations, and field occupational health education. Additionally, the PH officer or technician performs the following deployment operations.

5.2.3.1. In collaboration with BE, or in its absence, PH conducts an OEHSA at deployed locations, as outlined in DAFI 48-145, *Occupational and Environmental Health Program*. The OEHSA is documented in DOEHRS IAW AFTTP 3-2.82, *Occupational and Environmental Health Site Surveillance at Deployment Locations*, and OEHSA technical guidance. PH evaluates facility placement to identify potential health hazards, public health concerns, and sanitation issues.

5.2.3.2. Reviews all Deployment Medical Records of deployed personnel upon arrival to verify that all Force Health Protection requirements, such as medical countermeasures, Force Health Protection Prescription Products (FHPPPs), and vaccines, have been completed and/or issued. Addresses and mitigates any deficiencies identified during the review.

5.2.3.3. Monitors local area diseases by maintaining liaison with other preventive medicine activities within the theater, including host nation representatives.

5.2.3.4. Establishes and maintains liaison with other medical intelligence sources to ensure timely access to relevant information.

5.2.3.5. Develops and maintains a DNBI surveillance system tailored to the bed-down location to monitor and mitigate health risks.

5.2.3.6. Establishes an effective pest management system to ensure baseline vector surveillance is conducted promptly upon arrival at the deployed location. Identifies high-risk areas and provides recommendations for appropriate preventive and control measures. Coordinates with CE, if available, to implement integrated pest management control strategies. Advises the FS and deployed commander on the potential risks of vector-borne diseases. Re-educates and motivates all deployed personnel on the proper use of PPE to mitigate vector-related health risks.

5.2.3.7. Collaborates with Contracting and Services to ensure food is procured from safe, approved sources. If approved sources are unavailable, conducts FWRAs to evaluate alternative food sources and advise deployed commanders on the risks associated with their use for short durations.

5.2.3.8. Collaborates with Services personnel and food service staff to ensure proper storage, handling, and safety protocols are followed during food preparation and serving.

5.2.3.9. Verifies that all bottled water sources are approved and listed in the *Worldwide Directory of Sanitarily Approved Food Establishments for Armed Forces Procurement*.

5.2.3.10. Prevents foodborne and waterborne illnesses by implementing proactive measures, investigating outbreaks, providing food handler training, and recommending strategies to prevent future occurrences.

5.2.3.11. Collaborates with BE to ensure OEH prevention programs are effectively developed, implemented, and sustained throughout the deployment.

5.2.3.12. Investigates potential occupational illnesses or injuries and provides recommendations for appropriate preventive actions.

5.2.3.13. Controls disease transmission through patient interviews, epidemiological investigations, and targeted patient education. Advises healthcare professionals on effective control measures and ensures associated records and forms are updated and completed accurately. Investigates disease outbreaks, collects and compiles epidemiological data, and identifies, evaluates, and reports emerging trends.

5.2.3.14. If not already completed, strongly encourages all PH members to establish accounts on the NCMI and TRAVAX websites to access valuable resources and support mission requirements.

5.2.4. Independent Duty Medical Technician (IDMT). The IDMT's primary role is to collaborate with the FS as the only other team member capable of providing primary and urgent medical or dental care for deployed U.S. forces. The IDMT's scope of care is primarily limited to active-duty military personnel; however, they are authorized to provide acute and emergency care to anyone in situations involving risk to life, limb, or eyesight. IDMTs possess specialized training in mass casualty management and have limited training in BE and PH operations. Under the direction of BE or PH team members, they can assist with related tasks. In the absence of BE or PH personnel, IDMTs will perform all preventive medicine duties within their scope of training until BE or PH team members arrive.

5.3. Site/Airfield Survey. FFGRL members will provide the deployed commander and CE with health and safety recommendations regarding food, water, billeting, and hygiene. Additionally, FFGRL will conduct food and water vulnerability assessments and evaluate local infrastructure, such as medical facilities, lodging, and public works, to identify potential hazards or concerns.

5.4. Emergency Response. FFGRL members respond to emergencies and provide commanders with risk assessments to ensure the safety of responders. They also assist in developing hazardous material response plans, which should account for potential releases of TIC/TIM not only from DoD assets but also from the surrounding community. The FFGR1 medical equipment package offers minimal capabilities for sampling, analysis, and monitoring.

5.5. Medical Chemical, Biological, Radiological, Nuclear (CBRN) Operations. FFGRL/FFGR1 is not equipped to respond to CBRN incidents and has minimal capabilities for detecting and identifying CBRN hazards, limited to analysis of water. Despite these limitations, the team should maintain familiarity with counter-CBRN warfare concepts and utilize relevant medical intelligence to identify potential health hazards and recommend appropriate control measures.

5.6. Reporting. FFGRL will generate and distribute the following reports.

5.6.1. Situation Report (SITREP). In addition to daily CR SITREPs, a separate medical SITREP must be submitted to HQ AMC/SGX and the respective Command SGX office (e.g., PACAF, USAFE). The medical SITREP should briefly highlight key medical activities, issues, and trends related to personnel, as well as Medical Supplies/Blood (Class VIII/VIIIB) logistics. Submissions should be sent to HQ AMC/SGX at usaf.scott.amc-sg.mbx.amc-sgx-medical-plans-and-ops@mail.smil.mil and the appropriate Command SGX office. (Refer to Attachment 5 for a template)

5.6.2. The OEHSA will be reported in accordance with Defense Health Agency-Procedural Instruction (DHA-PI) 6490.03, *Deployment Health*, and submitted to the appropriate theater

Command BE for Quality Assurance (QA) review and verification. DOEHRS can be accessed at: https://doehrs-ih.csd.disa.mil/Doehrs. If connectivity is unavailable, data can be collected using documents available on the DOEHRS resources support website: https://ph.health.mil/topics/envirohealth/hrasm/Pages/DOEHRS_Resources.aspx. Once connectivity is restored, an electronic record should be created in DOEHRS, the collected data entered, and the original paper documents scanned and uploaded to the official DOEHRS record. No classified OEHSA information or data should be entered into DOEHRS. Any information that could render an OEHSA-related report classified must be submitted separately, following CCMD and service-specific guidance, to the Military Exposure Surveillance Library (MESL) at: oehs.data.army@mail.smil.mil.

5.6.3. Periodic Occupational and Environmental Monitoring Summary (POEMS). POEMS provides an overview of OEH exposure data and associated health risks for the deployed site. FFGRL will collaborate with the respective Air Component's SGP Office to determine whether POEMS generation or updates are required. With guidance from the Air Component's SGP Office and in accordance with DHA-PI 6490.03, *Deployment Health*, FFGRL will review and/or complete POEMS. POEMS will be shared with follow-on medical forces to support their completion of the OEHSA. **Note:** Cleared POEMS should not be included in individual DoD health records. Instead, they should be archived in DOEHRS and made accessible via the following website: https://ph.health.mil/topics/envirohealth/hrasm/Pages/POEMS.aspx.

5.6.4. Medical Site Assessment and Medical Laydown Plan (Site Surveys). FFGRL team lead will coordinate with the respective Air Component to identify any unique site survey documentation and reporting requirements. Once completed, site survey documents will be submitted to the respective Air Component's SGX or SGP office for review and recordkeeping.

5.7. Security. All aspects of Communications Security (COMSEC) and Operational Security (OPSEC) must be fully implemented and strictly enforced. During Force Protection Condition (FPCON) events, FFGRL will apply all necessary safeguards and may be required to provide physical security for their quarters and work areas. All team members are required to be dual weapons qualified with the M-18 and M-4 and must deploy with standard PPE as outlined in the OPORD.

Chapter 6

REDEPLOYMENT AND POST DEPLOYMENT

6.1. Overview. Upon notification of redeployment, the FFGRL team will initiate the repacking and preparation of all FFGR1 medical equipment assets for return to the home unit. The team will also compile all collected site information for reporting purposes and ensure it is effectively transferred to follow-on forces to support continuity of operations.

6.2. Redeployment Notification. Upon receiving redeployment notification, the CRE Commander will determine whether the movement involves redeployment to home station or forward deployment to an alternate location.

6.2.1. Redeployment to home station. Team members will conduct a thorough inventory and re-pack all FFGR1 medical equipment assets in accordance with the pack-out list. Hazardous items must be documented appropriately in compliance with DAFI 10-403, *Deployment Planning and Execution*. All personal gear must be packed exclusively in personal bags. Personal items must not be packed in FFGR1 asset containers under any circumstances.

6.2.2. Forward Deployment. FFGRL members will address and reconstitute any equipment shortages prior to deploying to the new site. The FFGRL team chief will communicate all shortfall information and provide recommendations to the deployed commander before the forward deployment. It is recommended that the deployed commander report any assemblage limiting factors to higher headquarters to ensure mission readiness and support.

6.3. Pre-Redeployment Data Transition. Regardless of the redeployment type, FFGRL members must ensure all relevant information collected at the deployed location is provided to follow-on medical forces as applicable. Key documents, including the OEHSA and any field inspection checklists, should be transferred to support ongoing environmental health surveillance efforts. Additionally, all applicable data should be thoroughly compiled, copied, and organized for continuity and future reference.

6.4. Post Deployment Reporting.

6.4.1. After Action Report (AAR). AARs serve as formal documentation of a unit's participation in the Chairman of the Joint Chiefs of Staff (CJCS) Exercise Program, other joint exercises, Air Force exercises, and real-world operations, including humanitarian missions, peacekeeping efforts, and Noncombatant Evacuation Operations (NEO). These reports are primarily prepared for review outside the medical unit and must be submitted in accordance with AFI 10-204, *Air Force Service Exercise Program and Support to Joint and National Exercise Program*. The senior FFGRL member is responsible for submitting an AAR to HQ AMC/SGX or the respective Command SGX office (e.g., PACAF, USAFE) within 30 days of returning to home station. Efforts should be made to complete the AAR prior to redeployment whenever possible. Before drafting the AAR, the author must ensure

the appropriate classification level is applied. AARs should provide sufficient detail and analysis to accurately account for events, including, at a minimum: dates, personnel involved, outcomes, recommended follow-up actions. **Note:** Prior to drafting an AAR, confirm the classification level with the classification authority for the deployment to ensure compliance.

6.4.2. It is critical that findings from AARs are entered into the JLLIS website at: https://www.jllis.mil/usaf. For entering information classified as SECRET, the FFGRL must access the JLLIS secure site via SIPRNet at: https://www.jllis.smil.mil/USAF. Registration is required to input information into the JLLIS platform.

6.4.3. Post Deployment Project Equipment Effectiveness Survey. The AAR should include a detailed report identifying any assets within the package that were ineffective during the deployment, along with an explanation of why and how they were not effective. Additionally, the report should highlight any assets deemed necessary or identified as new requirements that were not included in the AS. These items should be annotated in the AAR and submitted with clear justification. Ensure this portion of the AAR is forwarded to HQ AMC/SGX at usaf.scott.amc-sg.mbx.amc-sgx-medical-plans-and-ops@mail.smil.mil or the respective Command SGX office (e.g., PACAF, USAFE) within 30 days of returning to home station. The submitted information will be reviewed, and if determined to align with the Concept of Operations (CONOPS), it will be forwarded to the pilot unit for further evaluation.

Chapter 7

LOGISTICS

7.1. Overview. To maximize transportation flexibility, FFGRL deploys with a limited set of medical equipment designed for treatment, sampling, and analysis. FFGRL must deploy with the FFGR1 medical equipment package to ensure team members have all necessary supplies and capabilities to fulfill mission requirements.

7.2. FFGR1 Equipment. FFGR1 equipment UTC provides medical equipment and supplies to support initial preventive medicine capabilities and limited medical care for CRE personnel. FFGR1 is assigned to the CRE and is not allocated to a theater. **Note:** FFGR1 medical equipment AS requirements and pack-out guides are available on the Air Force Medical Logistics website: https://medlog.us.af.mil/areas/AFMedLog/#home.

7.3. Contingency Response Element (CRE) Support. The FFGRL team relies entirely on the CRE for transportation, billeting/tentage, communications support, SIPRNet access, food, fuel, and potable water at the deployed location.

7.4. Transportation. Transportation from home station to the deployment location may involve various modes. FFGRL orders will include authorization for excess baggage to accommodate mission-essential equipment and supplies.

7.5. Re-Supply, Reconstitution/Sustainment. FFGRL members will utilize organic units, such as Army MEDLOG Battalions, to provide re-supply capabilities. If supply support cannot be established, the team should contact the Air Force Medical Logistics Operations Center (AFMLOC) via: Phone: 301-619-2883 (regular hours) or 301-905-2697 (24/7 support) E-mail: usaf.detrick.afms.mbx.afmedcom-a4-10m-readiness-afmloc@health.mil. If the FFGR1 medical equipment package is expected to redeploy multiple times to assist in establishing several bases, the first base established should serve as the designated re-supply point. This approach prevents orders from being shipped to multiple locations. The first usable shipping destination will be used to establish a resupply pipeline. FFGRL personnel must coordinate with the first deployed base commander to create a forwarding protocol, ensuring assets are efficiently moved to the most current location in a timely manner.

7.6. Vehicles. Prior to arrival, vehicle support requirements and requests must be submitted by the FS to the deployed commander for approval. Vehicles may be obtained through local contracting efforts facilitated by the contracting office.

7.7. Patient Support. FFGRL may provide care for a limited number of patients who cannot be immediately evacuated. Temporary billeting, food, and potable water for these patients will be sourced from base operating support or other deployed resources to ensure their needs are met during their stay.

Chapter 8

INFORMATION MANAGEMENT

8.1. Overview. FFGRL is typically the first Health Service Support asset to arrive at a deployed location. The team deploys with the capability to initiate and manage data systems using various Joint Operational Medical Information Systems (JOMIS) software modules. All FFGRL members must maintain access to an active SIPRNet account, which must be established no later than six months after assignment to the UTC.

8.2. Information Assurance (IA) Policy. FFGRL members are required to understand and adhere to IA procedures in accordance with Department of the Air Force Manual (DAFMAN) 17-1301, *Computer Security (COMPUSEC)*, and other relevant Air Force IA guidance. Deploying FFGRL members must possess a current IA certificate to ensure compliance with security protocols and operational readiness.

8.3. Communication Systems.

8.3.1. Satellite/Local Area Network (LAN) Connectivity. Communication systems may be acquired upon arrival at the deployed location, depending on the existing infrastructure and the availability of commercial and/or military service-provided circuits.

8.3.2. Laptops. FFGRL deploys with standard laptops containing the DoD Standard Desktop Configuration (SDC) and JOMIS suite of products. Both are capable of word processing, database management, store-forward telemedicine, tele-maintenance, message text formatting, graphics, and LAN/Wide Area Network (WAN) interface capabilities.

8.3.3. Iridium Phones with Secure Sleeve. Iridium phones are handheld, worldwide satellite phones that are larger than typical mobile phones but compact enough to fit in a backpack. They are simple to use and provide secure/unsecure voice communication via satellite. For secure communications, a secure sleeve must be attached to the phone handset. Each unit will use their local Program Designator Code (PDC) to purchase and activate Subscriber Identity Module (SIM) cards for training and exercises. WRM SIM cards must not be used for unit training or exercises. When Iridium phones are deployed, the WRM SIM card will be activated using the appropriate contingency operation consolidated PDC. Activation can be completed via the following website: https://disa-storefront.disa.mil/dsf/logon?a=KINETIC&r=https://storefront.disa.mil/kinetic/disa/app/disa/authenticate/.

8.3.3.1. All relevant information for Iridium and International Marine/Maritime Satellite (INMARSAT) systems can be found at: https://www.inmarsat.com/en/solutions-services/maritime.html.

8.4. Software Applications. A standardized suite of products empowers commanders to track medical trends, implement preventive measures, and sustain force health by collecting,

analyzing, and disseminating medical information across services and throughout the theater in near real-time. For a detailed list of software applications, refer to Table 8.1: Software Applications.

8.4.1. Electronic Health Record (EHR) Capability. EHR capability extends electronic medical record functionality to the theater of operations, allowing healthcare providers to document patient care, order ancillary services such as blood labs, X-rays, and medications, and store medical data locally. This data is retained until communications are restored, at which point it can be transmitted to the Operational Medicine Data Store and Clinical Data Repository, ensuring continuity of care and accurate recordkeeping.

8.4.2. Defense Medical Logistics Standard Support (DMLSS). DMLSS is an automated and integrated information system that standardizes medical logistics management across military services. It provides a comprehensive suite of functions, including medical materiel management, equipment tracking, WRM assemblage oversight, and facilities management, ensuring efficient and standardized logistics operations in support of deployed medical teams.

8.4.3. Defense Occupational and Environmental Health Readiness System (DOEHRS). DOEHRS is a web-based portal designed to support exposure-based environmental surveillance for industrial health, environmental health, and hearing conservation communities. It enables capture, analysis, and maintenance of data on environmental and occupational hazards, facilitates the tracking of long-term exposures, and supports efforts to reduce worksite hazards through actionable insights.

8.4.4. Individual Longitudinal Exposure Record (ILER). ILER provides a longitudinal record of service-related exposures to improve medical, occupational, and environmental health surveillance and to detect emerging, latent health conditions. Available data includes CBRN, blast overpressure/traumatic brain injury, burn pit exposure, workplace exposures, and hazardous chemicals.

8.4.5. Joint Medical Workstation (JMeWS). JMeWS provides medical situational awareness, surveillance, and force health decision support. It enables reporting on medical trends, analyzing the overall status of theater health, and sharing medical intelligence with the Global Combat Support System (GCSS) and Global Command and Control System (GCCS). These capabilities enhance operational decision-making and provide commanders with a comprehensive view of theater health conditions.

8.4.6. Theater Medical Data Store (TMDS). TMDS provides unclassified web-based access to theater EHR information collected from deployed locations. Within TMDS, the Joint Patient Tracking Application (JPTA) serves as a URL-based tool that tracks the location and disposition of ill or injured patients as they progress through various echelons of care.

8.4.7. Patient Movement Item - Asset Tracking System (PMI-ATS). PMI-ATS is a military health system application designed to electronically track the locations and operational

statuses of medical equipment used to support patients during AE. PMI-ATS is accessible via a URL link within JOMIS, ensuring efficient management and accountability of critical medical assets.

8.4.8. TRANSCOM Regulating and Command and Control Evacuation System (TRAC2ES). TRAC2ES is a system used to manage global patient movement regulating and casualty evacuation processes by tracking patients exiting the theater via air. TRACE2S allows patient validation and regulation by USTRANSCOM to match patient requirements with en route medical care and assuring appropriate patient reception. TRAC2ES is accessible through a URL link within JOMIS, ensuring seamless integration with other medical systems.

8.4.9. DoD Platform One. DoD Platform One serves as a collaborative workflow tool that enhances visibility and coordination for FFGRL members during CR operations. It facilitates communication and operational awareness, ensuring effective collaboration across deployed teams.

Table 8.1. Software Applications

APPLICATION	PURPOSE
Health Assessment Light Operations (HALO) is replacing Armed Forces Health Longitudinal Technology Application – Theater (AHLTA-T) until JOMIS provides iOS capabilities.	Electronic Health Record (EHR) for theater environments
Defense Medical Logistics Standard Support (DMLSS)	Medical logistics management
Defense Occupational and Environmental Health Readiness System (DOEHRS)	OEH exposure tracking and surveillance
Individual Longitudinal Exposure Record (ILER)	Document service-related exposures
Joint Medical Workstation (JMeWS)	Medical C2
Theater Medical Data Store (TMDS)	Theater medical record database
Patient Movement Item Asset Tracking System (PMI-ATS)	AE certified PMI medical equipment tracking
TRANSCOM Regulating and Command and Control Evacuation System (TRAC2ES)	Medical regulating and patient validation
DoD Platform One	Collaboration with CR activities

8.5. Data Transition. FFGRL members are responsible for informing follow-on forces of all completed OEHSA activities and ensuring the appropriate transfer of data to support continuity of operations and ongoing environmental health surveillance.

8.6. Deployment Medical Records. Deployment medical records, including DD Form 2766 *Total Force Health Readiness Flowsheet*, must be handed over to medical personnel within 12 hours of arriving at the deployed location. Prior to redeployment, these records will be provided

to the deployed CRE commander, who will ensure they are delivered to the medical authority at the redeployment location. All records must be handled in accordance with DAFMAN 41-210, *Patient Administration Support*, to maintain proper documentation and compliance.

8.7. Help Desk Support. AMC/SG provides 24/7 help desk support, which can be accessed via the following telephone numbers and e-mail address:

DSN: 312-576-6472 (Central Standard Time duty hours)
Commercial: 618-256-6472 (duty hours)
On-Call Phone: 618-791-6700 (24/7)
E-mail: amc_ae_ehr@us.af.mil

Chapter 9

EDUCATION AND TRAINING

9.1. Overview. The FFGRL UTC plays a critical role in providing medical support to deployed forces. For each mission category, FFGRL must possess the necessary skills to seamlessly integrate with deployed forces and effectively support the deployed commander in achieving mission objectives.

9.2. Training Responsibilities. Home station Medical Readiness Office is responsible for scheduling all required mobility training. The FFGRL team chief, in coordination with the Medical Readiness Office, will ensure that all FFGRL-specific training requirements are completed to maintain readiness and operational effectiveness.

9.3. Formal Training. Given the vital health and preventive medicine services FFGRL members provide during the early stages of deployment—often in austere or bare base environments—they must participate in local training and exercises to remain proficient in all relevant contingency skills. FFGRL members are required to maintain currency in Comprehensive Medical Readiness Program (CMRP) training and review this TTP. Additionally, they must complete the Operational Preventive Medicine (OPM) course, the AFSC-designated Tactical Combat Casualty Care (TCCC) course, and the Evasion and Conduct After Capture course upon assignment to the FFGRL UTC, but no later than six months after assignment. These training requirements ensure FFGRL members are fully prepared to operate effectively in challenging deployment environments.

9.3.1. Operational Preventive Medicine (OPM). The OPM course is designed to prepare personnel assigned to UTCs whose mission includes field preventive medicine tasks during wartime contingencies, terrorist incidents, operations other than war, or natural disaster situations. It is particularly tailored for those assigned to the FFGRL UTC. Registration for the course can be completed at https://usafsam.geniussis.com/catalog/course/1216/operational-preventive-medicine-opm. The OPM course provides essential knowledge and training in areas such as bare base assets, preventive medicine readiness issues, personal protection from arthropods, arthropod-borne diseases of military significance, dangerous flora and fauna, field sanitation and hygiene practices, contingency contracting, field drinking water standards, ROWPU operations, field industrial hygiene procedures, outbreak investigation techniques, environmental health site assessments, characteristics of biological, chemical, and nuclear/radiological weapons, humanitarian operations, and after-action reporting. This comprehensive training equips personnel with the skills and knowledge necessary to address a wide range of preventive medicine challenges in austere and high-risk environments.

9.3.2. Evasion and Conduct After Capture (ECAC). Due to the potential operational locations and mission requirements, all FFGRL members are required to attend ECAC training. Attendance is scheduled through the Unit Training Manager. For personnel who have previously attended Survival, Evasion, Resistance, and Escape (SERE) courses, training exemptions may apply as outlined in AFI 10-202, *Contingency Response Forces*.

9.3.3. Tactical Combat Casualty Care (TCCC). All FFGRL members must be trained to the TCCC Tier required by their career field manager or consultant in accordance with the Air Force Medical Service (AFMS) Corps Chief TCCC Tier guidance. Recertification is required every 36 months following initial certification and within 12 months of deployment, as specified in DoD Instruction (DoDI) 1322.24, *Medical Readiness Training*. TCCC Tier 2 training is scheduled and completed through local MDGs, while Tier 3 and Tier 4 course seat requests are routed to MAJCOM TCCC Advisors.

9.3.4. Food and Water Risk Assessment (FWRA). The FWRA course provides standards for identifying foodborne and waterborne hazards and facilitates communication of associated health risks to deployed forces during missions where DoD-approved sources of food and water may not be available. Attendance is mandatory for 43HX Public Health officers assigned to the FFGRL.

9.3.5. Highly Recommended Training Opportunities. Additional recommended training includes the Medical Nuclear, Biological, and Chemical (NBC) Operations course, Medical Management of Chemical and Biological Casualties (MCBC) course, Military Tropical Medicine course, Operational Entomology course, and the Center for Sustainment of Trauma and Readiness Skills (C-STARS) course, which is particularly beneficial for FSs and IDMTs.

9.3.6. Other Training Opportunities. FFGRL members are encouraged to attend other relevant courses to enhance their preparedness for field preventive medicine operations. For example, the Air Advisor Course at the Expeditionary Center at JBMDL trains members on their roles and responsibilities in assessing the suitability and feasibility of a site during the deliberate planning phase of an operation. Additional recommended training includes courses offered by other military services, such as the U.S. Army chemical/biological warfare course and U.S. Navy preventive medicine course.

9.4. Equivalency Credit. FFGRL members may request or receive credit for real-world training and/or deployments. Members who have deployed for 14 days or more to an austere environment within a 48-month period and have performed duties consistent with those of an FFGRL or PAM member may request training exemptions. Exemption requests should be submitted to the respective Command's SGX, who will coordinate approval or disapproval with the Command's SGP.

9.5. Unit Level Training. The following hands-on training must be conducted at the unit level, provided the team has not received the training at formal courses: Cargo Preparation Pallet Build Up and Hazardous Material Preparers IAW DAFI 10-403, *Deployment Planning and Execution*, Global Positioning System (GPS), iridium satellite phone.

9.5.1. FS and IDMT, at a minimum, should receive training on TRAC2ES and HALO software.

9.5.2. Individual members must maintain currency in their CMRP training.

9.5.3. Vehicle and licensing requirements will mirror local CRE requirements.

9.5.4. Unit-level training should be refreshed annually unless otherwise specified.

9.6. Exercises. All personnel assigned to the FFGRL should train and exercise together and participate in exercise Turbo Distribution or similar alongside the CRE to practice base opening operations. This United States Transportation Command-led exercise requires Joint Task Force-Port Opening (JTF-PO) aerial port of debarkation forces to be ready to deploy within 12 hours of notification and positions them as among the first forces on the ground in support of the CCMD during the initial phases of humanitarian or contingency operations. All FFGRL members are required to attend Turbo Distribution or other MAJCOM-approved exercises. Exercise participation will be prioritized for FFGRL teams entering AFFORGEN PREPARE Phase to ensure readiness and operational effectiveness.

Table 9.1. FFGRL Training Requirements

TRAINING	POSITIONS
Review UTC TTPs, Mission Capability Statements (MISCAPs), Mission Essential Task Lists (METLs), and Allowance Standards (AS)	All FFGRL members
Operational Preventive Medicine (OPM)	All FFGRL members
Cargo Preparation Pallet Build-Up	Minimum of 2 personnel per FFGRL
Hazardous Material Preparers	Minimum of 2 personnel per FFGRL
Evasion and Conduct After Capture (ECAC)	All FFGRL members
Turbo Distribution (or other MAJCOM approved exercise)	All FFGRL members
Food and Water Risk Assessment (FWRA)	Public Health officers
TRANSCOM Regulating and Command and Control Evacuation System (TRAC2ES) and Health Assessment Light Operations (HALO) software	Flight Surgeon, Independent Duty Medical Technician
Tactical Combat Casualty Care (TCCC)	All FFGRL members
Global Positioning System (GPS)	All FFGRL members
Iridium satellite phone	All FFGRL members
M-18 and M-4 qualification	All FFGRL members

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Attachment 1

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Abbreviations and Acronyms

AAR – After Action Report
ABMSO – Accountable Base Medical Supply Officer
ACGIH – American Conference of Governmental Industrial Hygienists
ADVON – Advanced Echelon
AE – Aeromedical Evacuation
AFAFRICA – Air Forces Africa
AIR EVAC – Aeromedical Evacuation
AFDRSi – Air Force Disease Reporting System internet
AFFORGEN – Air Force Force Generation

AFI – Air Force Instruction
AF-IT – Air Force - Input Tool
AFMAN – Air Force Manual
AFMLOC – Air Force Medical Logistics Operations Center
AFMS – Air Force Medical Service
AFOSI – Air Force Office of Special Investigations
AFPMB – Armed Forces Pest Management Board
AFR – Air Force Reserve
AFRRI – Armed Forces Radiobiology Research Institute
AFSC – Air Force Specialty Code
AFTTP – Air Force Tactics, Techniques and Procedures
AHLTA-T – Armed Forces Health Longitudinal Technology Application – Theater
ALARA – As Low As Reasonably Achievable
AMC – Air Mobility Command
AMD – Air Mobility Division
ANG – Air National Guard
AOC – Air Operations Center
APECS – All Purpose Environmental Clothing System
APOD – Aerial Port of Debarkation
APOE – Aerial Port of Embarkation
ARC – Air Reserve Component
AS – Allowance Standard
ASIMS – Aeromedical Services Information Management System
ATWG – Anti-Terrorism Working Group
BAS&E – Base Support and Expeditionary
BE – Bioenvironmental Engineering
BEE – Bioenvironmental Engineer
BOS-I – Base Operating Support Integrator
BW – Biological Warfare
BWA – Biological Warfare Agent
C2 – Command and Control
C4 – Command, Control, Communication, and Computers
CASEVAC – Casualty Evacuation
CBRN – Chemical, Biological, Radiological and Nuclear
CCDR – Combatant Commander
CCMD – Combatant Command
CDC – Centers for Disease Control and Prevention
CE – Civil Engineering
CIA – Central Intelligence Agency
CJCS – Chairman of the Joint Chiefs of Staff
Class VIII/VIIIB – Medical Supplies/Blood
CMRP – Comprehensive Medical Readiness Program
COMAFFOR – Commander, Air Force Forces
COMAMC – Commander, Air Mobility Command

COMPUSEC – Computer Security
COMREL – Command Relationship
COMSEC – Communications Security
CONOPS – Concept of Operations
CR – Contingency Response
CRE – Contingency Response Element
C-STARS – Center for Sustainment of Trauma and Readiness Skills
CW – Chemical Warfare
CWA – Chemical Warfare Agent
DAF – Department of the Air Force
DAFI – Department of the Air Force Instruction
DAFMAN – Department of the Air Force Manual
DEPORD – Deployment Order
DHA-PH – Defense Health Agency – Public Health
DHA-PI – Defense Health Agency-Procedural Instruction
DIRMOBFOR – Director of Mobility Forces
DMLSS – Defense Medical Logistics Standard Support
DNBI – Disease and Non-Battle Injury
DoD – Department of Defense
DoDD – Department of Defense Directive
DoDI – Department of Defense Instruction
DOEHRS – Defense Occupational and Environmental Health Readiness System
DOEHRS-EH – Defense Occupational and Environmental Health Readiness System-Environmental Health
DOTMLPF-P – Doctrine Organization Training Materiel Leadership and Education Personnel Facilities-Policy
DRRS-S – Defense Readiness Reporting System-Strategic
DSN – Defense Switching Network
DTR – Defense Transportation Regulation
ECAC – Evasion and Conduct After Capture
ECS – Expeditionary Combat Support
EHR – Electronic Health Record
EMEDS – Expeditionary Medical Support
EPA – Environmental Protection Agency
ESOHSC – Environmental Safety and Occupational Health Service Center
EXORD – Execution Order
FBI – Foodborne Illness
FCG – Foreign Clearance Guide
FFGRL – Medical Global Reach Laydown Team
FFGR1 – Medical Global Reach Laydown Equipment
FGS – Final Governing Standards
FHPPP – Force Health Protection Prescription Products
FM – Field Manual
FOC – Full Operational Capability

FPCON – Force Protection Condition
FS – Flight Surgeon
FVA – Food Vulnerability Assessment
FWRA – Food and Water Risk Assessment
GCCS – Global Command and Control System
GCSS – Global Combat Support System
GPS – Global Positioning System
GS – General Schedule
HALO – Health Assessment Light Operations
HAZMAT – Hazardous Material
HRA – Health Risk Assessment
IA – Information Assurance
IAW – In Accordance With
ICAO – International Civil Aviation Organization
ICTB – Interfacility Credentials Transfer Brief
IDMT – Independent Duty Medical Technician
IE – Individual Equipment
IFE – In-Flight Emergency
IL4 – Impact Level 4
IMR – Individual Medical Readiness
INMARSAT – International Marine/Maritime Satellite
IOC – Initial Operational Capability
IOS – International Organization for Standardization
ISOS – International SOS
JBMDL – Joint Base McGuire-Dix-Lakehurst
JDDOC – Joint Deployment Distribution Operations Center
JLLIS – Joint Lessons Learned Information System
JMeWS – Joint Medical Workstation
JOMIS – Joint Operational Medicine Information Systems
JPTA – Joint Patient Tracking Application
JTF – Joint Task Force
JTF-PO – Joint Task Force–Port Opening
JWICS – Joint Worldwide Intelligence Communications System
LAN – Local Area Network
LNO – Liaison Officer
LOGDET – Logistics Detail
MAJCOM – Major Command
MCBC – Management of Chemical and Biological Casualties
MCR – Medical Capabilities Report
MEDRED-C – Medical Report for Emergencies, Disasters and Contingencies
MEDEVAC – Medical Evacuation
MEDLOG – Medical Logistics
MEFPAK – Manpower and Equipment Force Packaging System
MEG – Military Exposure Guidelines

MESL – Military Exposure Surveillance Library
METL – Mission Essential Task List
MFWS – Military Field Water Standards
MISCAP – Mission Capability Statement
MRA – MEFPAK Responsible Agency
MRE – Meal, Ready to Eat
MRL – Medical Resource Letter
MSAT – Medical Situational Awareness in the Theater
MTF – Medical Treatment Facility
NATO – North Atlantic Treaty Organization
NBC – Nuclear, Biological, and Chemical
NCDC – National Climatic Data Center
NCMI – National Center for Medical Intelligence
NEHC-TM PM – Navy Environmental Health Center Technical Manual, Occupational and Environmental Medicine Directorate.
NEO – Noncombatant Evacuation Operations
NIOSH – National Institute for Occupational Safety and Health
NIPRNet – Non-classified Internet Protocol Router Network
OCP – Operational Camouflage Pattern
OEBGD – Overseas Environmental Baseline Guidance Documents
OEH – Occupational and Environmental Health
OEHSA – Occupational and Environmental Health Site Assessment
OEHSS – Occupational and Environmental Health Site Surveillance
OPCON – Operational Control
OPM – Operational Preventive Medicine
OPORD – Operation Order
OPR – Office of Primary Responsibility
OPREP – Operational Reporting
OPSEC – Operational Security
PACAF – Pacific Air Forces
PAM – Preventive and Aerospace Medicine
PAR – Population At Risk
PDC – Program Designator Code
PDHA – Post Deployment Health Assessment
PH – Public Health
PLANORD – Planning Order
PLHA – Preliminary Hazard Assessment
PMI-ATS – Patient Movement Item - Asset Tracking System
POC – Point of Contact
POEMS – Periodic Occupational and Environmental Monitoring Summary
PPC – Personnel Processing Codes
PPE – Personal Protective Equipment
PT – Physical Training
QA – Quality Assurance

R&EM – Readiness and Emergency Management
ROWPU – Reverse Osmosis Water Purification Unit
SDC – Standard Desktop Configuration
SERE – Survival, Evasion, Resistance, and Escape
SG – Surgeon General/Command Surgeon
SGP – Aerospace Medicine Division
SGX – Operational Medical Readiness Division
SIM – Subscriber Identity Module
SIPRNet – Secret Internet Protocol Router Network
SITREP – Situation Report
SOP – Standard Operating Procedures
SPOE – Sea Port of Embarkation
TCCC – Tactical Combat Casualty Care
TIC/TIM – Toxic Industrial Chemical/Toxic Industrial Material
TIR – Toxic Industrial Radiological
TMDS – Theater Medical Data Store
TPMRC – TRANSCOM Patient Movement Requirements Center
TRAC2ES – TRANSCOM Regulating and Command and Control Evacuation System
TTP – Tactics, Techniques, and Procedures
USAF – United States Air Force
USAFE – United States Air Forces Europe
USAMRICD – United States Army Medical Research Institute of Chemical Defense
USAMRIID – United States Army Medical Research Institute of Infectious Diseases
UTC – Unit Type Code
VSIMS – Veterinary Service Information Management System
WAN – Wide Area Network
WARNORD – Warning Order
WG – Wage Grade
WRM – War Reserve Materiel

Attachment 2

FLIGHT MEDICINE RECOMMENDED REFERENCES

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Forms

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- DD Form 2766, *Total Force Health Readiness Flowsheet* https://www.esd.whs.mil/Directives/forms/dd2500_2999/DD2766/
- DD Form 2795, *Pre-Deployment Health Assessment* https://www.esd.whs.mil/Directives/forms/dd2500_2999/DD2795/
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Attachment 3

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Forms

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Attachment 5

EXAMPLE SITREP

MEDICAL DAILY SITREP	[insert location]
1. Go Pill Usage (how many personnel requested/received them)	
2. No-Go Pill Usage (how many personnel requested/received them)	
3. Aircraft/Ground Mishaps (give details/ensure notifications were made)	
4. Total patients seen	
4a. Placed on quarters or limited duty	
5. Patients Seen at Civilian Medical Facilities (total #/was ISOS contacted and notified)	
5a. Placed on quarters or limited duty	
5b. Admissions/Inpatient (send separate e-mail with patient details)	
6. 9-line notification for MEDEVAC (give details/send separate e-mail w/patient details)	
7. TPMRC notification for AIR EVAC (give details/send separate e-mail w/patient details)	
8. Fatality (immediate notification MUST be completed)	
9. Identifiable Trends (FBI, DNBI, Infectious Disease, etc.—give details)	
9a. Foodborne Illness	
9b. Infectious Disease	
9c. DNBI	
10. Water testing completed (i.e. bottled water, water distribution system & frequency)	
11. Sanitation concerns	
12. Additional comments (include issues/concerns over last 24 hours and next 24 hours)	Last 24: Next 24:

Attachment 6

OPERATING CAPABILITY CONDITIONS

A6.1. Initial Operational Capability (IOC). The FFGRL team can reach IOC within 24 hours of arrival. IOC is reached when the following tasks have been completed:

A6.1.1. FFGR1 has been delivered to the site and is accessible by the team.

A6.1.2. Capable of essential and emergent care.

A6.1.3. Established communication with the deployed commander.

A6.1.4. IFE coverage is arranged and plan established.

A6.1.5. OEHSA has begun, including input into site selection and layout of facilities.

A6.1.6. Medical tent/facility is determined, and set-up has begun.

A6.1.7. Attendance to the daily Operations Center huddles and initiated medical briefings (e.g. location of medical tent, flora/fauna brief, medical emergency plan, etc.).

A6.2. Full Operational Capability (FOC). The FFGRL team can reach FOC within 48 hours of arrival. FOC is reached when the following tasks have been completed:

A6.2.1. Medical tent/facility is fully established, and equipment is available for operational use.

A6.2.2. FFGR1 has been inventoried.

A6.2.3. Transportation means established to move patients and conduct OEH surveys.

A6.2.4. Communications are established with the local C2, local health care authorities (e.g. Joint, coalition, civilian), theater surgeon, and line assets (including Army/Navy) at the bed-down location.

A6.2.5. Contact and establish access to the MEDEVAC (helicopter/9-line) and AIR EVAC (TPMRC) system.

A6.2.6. Local medical capabilities have been determined. Casualty Evacuation CASEVAC (ground) plan and referral procedures are established.

A6.2.7. A safe source of food and potable water is established.

A6.2.8. Deployment Medical Records (DD Form 2766, *Total Force Health Readiness Flowsheet*) have been received and reviewed.

A6.2.9. TIC/TIM and food and water vulnerability assessments are initiated.

A6.2.10. Patient encounter data is being documented, trends identified and forwarded to the theater surgeon, as needed.

A6.2.11. Capable of detection and tracking of potential exposures of harmful agents, OEH sample results, and information on activities.

A6.2.12. Establish work shifts.

A6.2.13. Mass Casualty plan established and briefed (including casualty collection point).

A6.2.14. Aircraft Mishap Response plan established and briefed (be aware of how many aircraft are coming in and out each day and type).

Attachment 7

CONTINGENCY RESPONSE FORCE PACKING LIST AND EQUIPMENT

A7.1. Packing Considerations. The following packing list was copied from the Contingency Response: Core Capabilities website: https://usaf.dps.mil/teams/TTP/SitePages/CR-Core.aspx

- Mandatory Items
  - A-Bag: Helmet, Canteen or CamelBak, Sleeping bag/pillow
  - Gas mask/ Training Chemical Gear
  - PPE: Gloves, Reflective Belts, Eye and Hearing protection, steel-toed boots, etc.
  - 2 sets of OCPs
  - PT Gear (Civilian clothes authorized)
  - Socks/underwear/undershirts
- Hand Carried Items
  - IDs: Military ID, Dog Tags
  - Government Driver's License
  - Flight Line Badge
  - Mobility Folder
  - Interfacility Credentials Transfer Brief (ICTB) for provider and IDMT only
- Recommended
  - Sunscreen/Sunglasses
  - GoreTEX/Wet Weather Gear
  - Towel
  - Shower Shoes
  - Running Shoes
  - Knife/Leatherman
  - Baby Wipes
  - Hygiene Kit: Toothbrush/Toothpaste, Deodorant, Shaving Kit, Wipes, etc.
  - Self-Care Kit: Band-Aids, Tweezers, Cough Drops, Ibuprofen, Electrolytes
  - Bug Repellent - See UDMs if you would like permethrin for your uniforms
  - Flashlight
  - Coffee mug
- 72-Hour Bags
  - Helmet
  - Vest
  - Sleeping bag/pillow
  - 2x MREs
  - PPE as required

A7.2. Individual Equipment. The Individual Equipment (IE) items listed below are recommended for the FFGRL personnel upon entering activation cycle to be in line with the IE utilized by the CREs.

Table A7.1. CRE IE

ITEM	QUANTITY
Work Gloves, Black or Coyote (e.g. Mechanix M-Pact)	1
Multi-Purpose Work Gloves (e.g. Original Mechanix Wear - thin)	1
Multi-Tool	1
Noise Reduction Hearing Protection (e.g. Peltor 3M tactical)	1
LED Headlamp	1
Flashlight	1
Sunglasses	1
Hydration Backpack (Coyote/OCP)	1
Duffle Bag (e.g. 5.11 SOMS or S.O.C. Loadout XL)	1
72-Hour Assault Pack, Coyote/OCP	1
Large Screw-On Carabiner	1
Reflective Belt	1
Dry/Compression Sack	4
Sleeping Bag (e.g. Softie 3 Merlin)	1
Sleeping Pad – Inflatable	1
Coyote Fleece Cap	1
Massif Extreme Weather Overhead Cover Hood	1
Coyote Steel Toe/Safety Toe Boots	1
Lightweight Cold Weather Shirt (Level 1)	2
Lightweight Cold Weather Bottoms (Level 1)	2
Heavyweight Waffle Top (Level 2)	2
Heavyweight Waffle Bottom (Level 2)	2
Massif Strato Low Loft Jacket	1
OCP APECS Jacket (GORE-TEX)	1
OCP APECS Pants (GORE-TEX)	1
OCP 3-Piece Rain Suit, Hot-Weather	1
All-Weather Socks	6
Massif Combat Pants w/ Knee Pads	2
OCP Combat Shirt (Army Combat Shirt)	3
US Flag Patch	2
OCP Rank Insignia	2

Attachment 8

EXAMPLE SITE SURVEY CHECKLIST

The document aims to collect data from the deployment area to generate a pre-deployment health assessment.

- All Elements Yellow, Blue, and Gray have some questions that can be answered with the help of online resources. The checklist must be reviewed **in its entirety before departure to answer as many questions as possible from All Elements Yellow, Blue, and Gray.** Helpful links are on the resources page of this document.
- Elements in **Yellow** are the minimum requirements for any assessment bare base or an established location. While in the deployment country, these should be the primary focus and answered entirely.
- Elements in **Blue and Gray** are to be completed boots on the ground if time permits or if there is an imminent health hazard or threat to the mission.
- Classification of the checklist becomes **CUI to Secret upon completion**.

OPR /OCR:	
Phone:	
E-mail:	
Date:	
Site name:	
Site code - International Civil Aviation Organization (ICAO):	
Site location (grid coordinates):	
Surveyor(s) and unit assignment:	
Languages spoken in the area:	
Interpreter name(s):	
Medical emergency number (i.e., 911) and/or who to call:	
Are there Foreign Clearance Guides (FCG) (e.g., Covid-19, etc.) guidance requirements? Provide link to FCG.	

DATA ELEMENT	PRIORITY
Base Support	**Tier 1: Basic requirements for any location threat**
Host Nation Capabilities and Medical Evacuation	**Tier 2: If time and location permit**
Advanced Assessments and Threats	**Tier 3: If time/capacity permit and/or threat required**

*****Note: Mark N/A for questions that DO NOT apply.**

1.1. BASE SUPPORT	
1. What are the lodging / billeting provisions and plans?	
1.1. Are critical bed-down facilities/functions located in a flood zone?	☐ Yes ☐ No
1.2. Are there counter-flood contingency plans in place?	☐ Yes ☐ No
2. Is the bed occupancy-to-sleeping quarter ratio acceptable (beds at least 30 inches apart)?	☐ Yes ☐ No **How Many?**
2.1. Are sleeping quarters clean, free of debris and pests?	☐ Yes ☐ No
2.2. Is there presence of bedbugs in living / sleeping quarters?	☐ Yes ☐ No
3. Are sleeping quarters away from the flightline (e.g., noise, fuel fumes, etc.)?	☐ Yes ☐ No
4. Are appropriate types of toilets/latrines used at the site (e.g., straddle trench, deep trench, etc.)? (Yes/No, type?) Number of latrines (#) *Skip to 5 if hardened facilities	☐ Yes ☐ No **How Many?** **Type:**
4.1. Are latrines at least 100 yd downhill, downwind, and downstream from food facility?	☐ Yes ☐ No ☐ N/A
4.2. Are latrines at least 100 ft from nearest drinking water source?	☐ Yes ☐ No ☐ N/A
4.3. Are latrines at least 50 ft from sleeping quarters?	☐ Yes ☐ No ☐ N/A
5. Do showers have a soakage pit underneath them to prevent water from collecting and forming pools?	☐ Yes ☐ No ☐ N/A
6. Describe hand-washing areas, # of showers, and latrine facilities (i.e., building structure, cleanliness, stocked bathrooms, working sinks, ventilation, plumbing, etc.).	
7. Does food facility have a grease trap between the facility and soakage pit? *Skip to 8 if No or N/A	☐ Yes ☐ No ☐ N/A
7.1. Is food facility located 100 yds uphill, upwind, upstream from latrines?	☐ Yes ☐ No ☐ N/A
7.2. Is food facility 30 yds from garbage and soakage pits?	☐ Yes ☐ No ☐ N/A
7.3. Is food facility a minimum of 50 ft from latrines?	☐ Yes ☐ No ☐ N/A
7.4. Is food facility away from the flightline?	☐ Yes ☐ No ☐ N/A
8. Are drinking water access points located upstream and at least 50 ft from all waste sites and sewage disposal system?	☐ Yes ☐ No ☐ N/A
9. Is clinic/hospital located as far away as possible from tactical targets (i.e. fuel tanks, headquarters bldg., flightline)?	☐ Yes ☐ No ☐ N/A
10. Is patient decontamination site located 250 ft downhill, downwind, downstream from the hospital, and at an accessible location to facilitate patient flow?	☐ Yes ☐ No ☐ N/A
11. Is the contaminated materials disposal site located at least 75 ft downhill, downwind, and downstream from the patient decontamination site?	☐ Yes ☐ No ☐ N/A
12. Identify solid waste disposal process (e.g., municipal collection program, incinerators, etc.).	
13. Are there any burn pits or industrial pollution/threats on site or within 10 miles?	☐ Yes ☐ No
14. Are there any incinerators on site?	☐ Yes ☐ No
15. Describe the regulated medical waste (e.g., needles, blood-soaked items, etc.) disposal process.	
16. Identify and assess buildings of opportunity (i.e., structural stability, water, toilets, etc.).	
17. Are public facilities available for personnel use (e.g., mini mall, hair salon/barber shop, laundry, etc.)?	☐ Yes ☐ No **LIST:**

***Skip to 18 if No**	
17.1. Are facilities in good repair & clean?	☐ Yes ☐ No
17.2. Is there a cleaning / maintenance schedule?	☐ Yes ☐ No
17.3. Who cleans / restocks the facility? If the service is contracted, provide company information.	
17.4. If cleaning services are not provided, are cleaning, and sanitizing materials available?	☐ Yes ☐ No
17.5. Are the facilities heated, cooled, and / or ventilated?	☐ Yes ☐ No
17.6. Are garbage, refuse, and other solid waste disposed of in an acceptable manner?	☐ Yes ☐ No
17.7. Are the facilities relatively free of vector (e.g., rats, birds, mosquitos, etc.) breeding areas?	☐ Yes ☐ No
17.8. Public facility POCs:	
18. If laundry facilities are available:	
18.1. Specify # of washers, # of dryers.	**Washers: Dryers:**
18.2. Is access limited?	☐ Yes ☐ No
18.3. If hotel linen laundry is contracted out, provide name of the company.	
18.4. If personal laundry service is available, LIST hotel or provide name of the company.	**LIST:**
19. Is there potential space to set up at ROLE 1?	☐ Yes ☐ No
20. Attach force health protection strategies for identified disease threats to checklist, LIST data sources.	**LIST:**
21. Additional Base Support POCs and Phone Numbers:	
22. Additional Comments:	

1.2. HOST NATION WATER	
1. Is Host Nation water available / utilized as a drinking water source?	☐ Yes ☐ No
2. Is on site water (i.e., wells, surface, etc.) being utilized as a drinking water source? LIST *Skip to 3 if No	☐ Yes ☐ No **LIST:**
2.1. Is Installation drinking water mixed with Host Nation water?	☐ Yes ☐ No
3. Are there any complaints about drinking water taste/smell or inconsistent qualities that may need investigating? LIST	☐ Yes ☐ No **LIST:**
4. Has AF Bioenvironmental Engineering (or equivalent sister service Prev Med professionals) conducted a Water Vulnerability Assessment?	☐ Yes ☐ No
5. Has a sanitary survey been conducted in last 5 years? *Skip to 6 if No or N/A	☐ Yes ☐ No ☐ N/A
5.1. What is the date of last sanitary survey? Who conducted it?	**Date:**
6. Are drinking water system managers/operators aware of or utilizing the Host Nation-specific Final Governing Standards (FGS)?	☐ Yes ☐ No ☐ N/A
6.1 If No, what is being used [i.e., DoD, AF, MFWS (Short/Long term), Army Tech Guide 230 MEGs, NATO, OEBGD or other]? LIST *Skip to section 7 if sanitary survey is not being accomplished.	**LIST:**
7. From current sanitary survey report: How many significant deficiencies related to operations and management have been identified?	**Number of deficiencies:**

7.1. How many significant deficiencies related to drinking water source condition and protection from pollution?	**Number of deficiencies:**
7.2. How many significant deficiencies related to inadequate drinking water treatment?	**Number of deficiencies:**
7.3. How many significant deficiencies related to proper storage of finished water for drinking or meal prep?	**Number of deficiencies:**
7.4. How many significant deficiencies related to infrastructure (distribution/pumps, well heads, etc.)?	**Number of deficiencies:**
7.5. How many significant deficiencies related to water quality monitoring?	**Number of deficiencies:**
7.6. Which deficiencies create an acute/immediate health hazard if used for drinking and/or meal prep?	
8. Is a sampling plan or equivalent being followed and in compliance with FGS?	☐ **Yes** ☐ **No** ☐ **N/A**
9. Is microbiological testing being conducted? *Skip to 10 If No or N/A	☐ **Yes** ☐ **No** ☐ **N/A**
9.1. Are the individuals conducting microbiological testing trained AF Prev Med AFSCs (i.e., BE, PH, IDMT, FS) or equivalent sister service or Host Nation Prev Med professionals?	☐ **Yes** ☐ **No** ☐ **N/A**
9.2. Does drinking water meet microbiological potability standards in TB MED 577 Table 4-3?	☐ **Yes** ☐ **No** ☐ **N/A**
10. Does the drinking water monitor have appropriate contact information and reach-back relationship with MTF Prev Med team that has BOS-I and/or Service component Subject Matter Expert?	☐ **Yes** ☐ **No** ☐ **N/A**
11. Does Prev Med testing/sampling team have adequate supplies and resources (e.g., media, bottles, incubator, etc.)?	☐ **Yes** ☐ **No** ☐ **N/A**
12. Are FGS sampling parameters and frequencies being met?	☐ **Yes** ☐ **No** ☐ **N/A**
13. Are all FGS baseline water monitoring results (e.g., physical, disinfection byproducts, inorganics, organics, pesticides, radiological, etc.) complete and available for review?	☐ **Yes** ☐ **No** ☐ **N/A**
14. Who is conducting monitoring? Provide contact information. (LIST Names; Phone number, etc.)	
15. Additional Comments:	

1.3. BOTTLED WATER SOURCE	
1. Is bottled water being utilized as a drinking water source?	☐ **Yes** ☐ **No**
2. Are there any complaints about water taste, smell or inconsistent qualities that may need investigating? LIST	☐ **Yes** ☐ **No** **LIST:**
3. Is bottled water being utilized for washing dishes, cooking, meal prep?	☐ **Yes** ☐ **No**
4. What is the bottled water source (e.g., location, bottling plant, etc.)?	
5. Who are the suppliers?	**LIST:**
6. Is supplier approved by DHA-PH (veterinary service personnel) and listed in the *Worldwide Directory of Sanitarily Approved Food Establishments for Armed Forces Procurement*? *Skip to 7 if No	☐ **Yes** ☐ **No**
6.1 When was last inspection from approved source? Date	**Date:**
7. Are adequate measures in place to ensure water was not tampered with during delivery? LIST measures.	☐ **Yes** ☐ **No** **LIST:**
8. Is microbiological testing being conducted on each water bottle lot IAW TB Med 577?	☐ **Yes** ☐ **No**

9. Are trained and qualified AF Medics (e.g., BE, PH, IDMT, FS) or equivalent sister service Prev Med professionals conducting sampling testing to include oversight?	☐ Yes ☐ No
10. Does drinking water monitor have appropriate contact information and reach-back relationship with MTF Prev Med team that has BOS-I and/or service component Subject matter Expert?	☐ Yes ☐ No
11. Does Prev Med testing/sampling team have adequate supplies and resources (e.g., media, bottles, incubator, etc.)?	☐ Yes ☐ No
12. Are there any other concerns with testing / sampling parameters or results? LIST	☐ Yes ☐ No **LIST:**
13. Bottled water storage:	
13.1. Is bottled water stored away from direct sunlight and off the ground?	☐ Yes ☐ No
13.2. Is bottled water stored at a temperature between 50°-70°F?	☐ Yes ☐ No
13.3. Is bottled water stored away from solvents and chemicals such as gasoline, paint thinners, household cleaners, and dry-cleaning chemicals?	☐ Yes ☐ No
14. Who is conducting monitoring? Provide contact information.	**LIST:**
15. Additional Comments:	

1.4. FOOD AND SECURITY	
1. Outline food provisions and plans (If MREs are the ONLY food source allowed for the entire duration of the deployment, answer 6 ONLY)	
2. Identify the approval authority for food: Worldwide Directory, FWRA or local approval? (FWRA or local approval must be conducted by trained/qualified Public Health or equivalent sister service Prev Med professionals)	
4. Has AF Public Health (or equivalent sister service Prev Med professionals) conducted an FVA?	☐ Yes ☐ No
5. Are there adequate eating facilities to support deployment, including equipment, personnel, and handwashing facilities for patrons?	☐ Yes ☐ No
6. Are food storage areas adequate for perishable, non-perishable and MRE Storage? LIST locations	☐ Yes ☐ No **LIST:**
7. Do local food services personnel follow adequate sanitary practice standards?	☐ Yes ☐ No
8. Will the meal preparation facilities meet US standards?	☐ Yes ☐ No
9. Will food handlers meet US standards?	☐ Yes ☐ No
10. Is the disposal method of garbage / waste at site acceptable (e.g., burial, incineration, etc.)?	☐ Yes ☐ No **Method:**
11. How are food products transported to the site? LIST mode of transportation and temperature conditions.	**LIST:**
12. Describe the local food inspection system (e.g., who inspects, effectiveness, etc.)	
13. Attach Force Health Protection strategies to ensure food safety and defense. LIST sources.	**LIST:**
14. Are DoD Prev Med professionals assigned to inspect DoD food facilities?	☐ Yes ☐ No
14.1. POCs Names and Phone Numbers	
15. Additional Comments:	

2.1. HOST NATION MEDICAL CAPABILITIES	
1. Name of the Host Nation hospital service:	
2. Is this location a community hospital, regional medical center or equivalent?	
3. Who surveyed the facility? (i.e., ISOS/TRICARE)	
3.1. Contact DSN and Commercial:	
3.2. Check the Medical Capabilities Report (MCR) and attach the updated ISOS/TRICARE hand out. (Send MCR Request to the regional TRICARE Office) https://www.tricare.mil/ContactUs/CallUs/OverseasResources TAO-EA (CENTCOM/EUCOM/AFRICOM) E-mail: usarmy-tao-ea@health.mil TAO-TLAC (SOUTHCOM/NORTHCOM/SOCOM) E-mail: dha.taolac@health.mil TAO-PA (PACOM) E-mail: dha-tao-pacific@health.mil Requests for MCRs may also be made to the National Center for Medical Intelligence (NCMI), as follows: National Center for Medical Intelligence E-mail: ~DIANCMIOPS@dodiis.mil ncmi_ops@dia.smil.mil Webpage: https://www.dia.smil.mil	☐ **Attached** ☐ **N/A**
4. Does facility appear in TRAVAX? https://private.travax.com/resources/)	☐ **Yes** ☐ **No**
5. Review and attach the MCR/ISOS Report	☐ **Yes** ☐ **No** **LIST:**
6. Location	
6.1. POCs	
6.2. Phone numbers	
6.3. Hours of Operation	
7. Distance to the airfield	
7.1. Does it have Helicopter PAD?	☐ **Yes** ☐ **No**
8. Ambulance Service	☐ **Yes** ☐ **No**
9. Attach Map of the facility	
10. Does the facility accept TRICARE?	☐ **Yes** ☐ **No**
11. Inpatient capability? # of beds?	☐ **Yes** ☐ **No** **Number of Beds:**
12. Critical care capability? # of beds?	☐ **Yes** ☐ **No** **Number of Beds:**
13. Is there an outpatient clinic?	☐ **Yes** ☐ **No**
14. Trauma center?	☐ **Yes** ☐ **No**
15. Blood Supply?	☐ **Yes** ☐ **No**
15.1. Does blood collected meet International Organization for Standardization (IOS) standards?	☐ **Yes** ☐ **No**
15.2. Is there a blood bank?	☐ **Yes** ☐ **No**

15.3. What is the blood capacity:	
15.4. Blood types available:	
16. General surgery capabilities?	☐ **Yes** ☐ **No**
17. Neurosurgery?	☐ **Yes** ☐ **No**
18. Orthopedics?	☐ **Yes** ☐ **No**
19. Internal Medicine?	☐ **Yes** ☐ **No**
20. Dental?	☐ **Yes** ☐ **No**
21. Immunizations Clinic?	☐ **Yes** ☐ **No**
22. Radiology (X-Ray, CT scan, MRI)?	☐ **Yes** ☐ **No** **LIST:**
23. Burn Specialist?	☐ **Yes** ☐ **No**
24. Cardiac Cath Lab?	☐ **Yes** ☐ **No**
25. Pharmacy?	☐ **Yes** ☐ **No**
26. Hospital Linen Service? * Skip to 27 If No	☐ **Yes** ☐ **No**
26.1. Medical laundry options: (In house/contract)	
26.2. Name of laundry contractor and frequency	
26.3. How is dirty/soiled linen handled and stored?	**LIST:**
26.4. Are towels and linens cleaned, transported, and stored adequately?	☐ **Yes** ☐ **No**
27. Hazmat/NBC response capability?	☐ **Yes** ☐ **No**
28. What types of supplies are necessary for an influx of military personnel?	**LIST:**
28.1. Does the facility have a resupply vendor in the local area for the supplies?	☐ **Yes** ☐ **No**
29. Does the facility have an expansion plan?	☐ **Yes** ☐ **No**
29.1. If Yes, how many beds are added?	**Number of Beds:**
29.2. Does the facility have access to interpreters?	☐ **Yes** ☐ **No**
30. Is the in-country medical supply chain established? (ONLY applies to U.S. MTF) *Skip to 31 if N/A	☐ **Yes** ☐ **No** ☐ **N/A**
30.1. LIST 5Ws for primary and secondary supply chain.	**LIST:**
30.2. LIST Line of accounting and customer account	**LIST:**
30.3. How are medical supplies delivered?	
30.4. Where are medical supplies delivered?	**LIST:**
30.5. What is the narcotic re-supply and issuance procedure?	**LIST:**
30.6. How are perishable medical items stored and transported?	**LIST:**
31. Additional comments:	

2.2. EVACUATION CAPABILITIES	
1. Medical evacuation system: U.S. Military or civilian?	
1.1. Method of payment (i.e., Local currency, USD, card, TRICARE)	**LIST:**
1.1.1. POC and Phone number and e-mail.	**LIST:**
2. Landing zone? If Yes LIST coordinates.	☐ **Yes** ☐ **No** **Coordinates:**
3. Closest airfield (APOE)?	
4. Closest sea port (SPOE)?	
5. For both questions 3. and 4. Answer the following	
5.1. Coordinates?	
5.2. Procedures?	
5.3. POC(s)?	
5.4. Phone numbers?	
5.5. Projected time to APOD?	
5.6. Describe the terrain	
5.7. Is there security?	☐ **Yes** ☐ **No** ☐ **N/A**
6. Fire department crash/rescue: military or civilian?	
6.1. Phone numbers:	
7. Host Nation ambulance: miliary or civilian?	
7.1. Mode: Air, sea, land?	
7.1.1. If Air, is there a country clearance to fly through airspace? (i.e., point of origin to destination)	☐ **Yes** ☐ **No** ☐ **N/A**
7.2. Does patient have to be stable to be evacuated?	☐ **Yes** ☐ **No** ☐ **N/A**
7.3. Select from the options below the type of evacuation transportation: ambulance, aircraft, ship, vehicle of opportunity	
7.3.1. Ambulance	☐ **Yes** ☐ **No** ☐ **N/A**
7.3.1.1. Capacity # of passengers litter and ambulatory?	
7.3.1.2. Vehicle dimensions (size, weight, etc.)?	
7.3.1.3. Radio Communications?	☐ **Yes** ☐ **No**
7.3.1.4. Medical Supplies on Board	☐ **Yes** ☐ **No**
7.3.1.5. Is oxygen on board	☐ **Yes** ☐ **No**
7.3.2. Aircraft	☐ **Yes** ☐ **No** ☐ **N/A**
7.3.2.1. Capacity # of passengers litter and ambulatory?	
7.3.2.2. Aircraft dimensions (size, weight, etc.)?	
7.3.2.3. Radio Communications?	☐ **Yes** ☐ **No**
7.3.2.4. Medical Supplies on Board	☐ **Yes** ☐ **No**
7.3.2.5. Is oxygen on board	☐ **Yes** ☐ **No**
7.3.2.6. Is aircraft pressurized?	☐ **Yes** ☐ **No**
7.3.2.7. Is there hoist capability if rotary wing?	☐ **Yes** ☐ **No** ☐ **N/A**
7.3.3. Ship / Vessel	☐ **Yes** ☐ **No** ☐ **N/A**
7.3.3.1. Capacity # of passengers litter and	

ambulatory?	
7.3.3.2. Vessel dimensions (size, weight, etc.)?	
7.3.3.3. Radio Communications?	☐ Yes ☐ No
7.3.3.4. Medical Supplies on Board	☐ Yes ☐ No
7.3.3.5. Is oxygen on board?	☐ Yes ☐ No
7.3.3.6. Is there a pressurized room in the vessel?	☐ Yes ☐ No
8. Additional comments:	

3.1. DISEASE SURVEILLANCE	
1. Are there endemic diseases in the area?	☐ Yes ☐ No
2. LIST any vector-borne and zoonotic disease present.	
3. LIST any food/waterborne diseases present.	
4. LIST any infectious diseases present.	
5. Proximity of MTF and/or main camp to sources of disease threats?	
6. Is there a mechanism for disease surveillance?	☐ Yes ☐ No
7. Describe disease surveillance methodology and reporting processes.	
8. Public Health reach back POCs and phone numbers?	
9. Outline Force Health Protection strategies for identified disease threats.	
10. LIST the immunizations, medications, labs, and PPE recommended for this site. (e.g., Typhoid vax, antimalarial pills, TB test, BW/CW, etc.)	
11. Additional comments:	

3.2. ENVIRONMENTAL HEALTH	
1. Are there plans for future industrial activities that could create an exposure pathway or any high risk industrial processes (e.g., corrosion control, etc.)?	☐ Yes ☐ No
1.1. If Yes, provide details.	
2. Has an initial OEHSA been completed?	☐ Yes ☐ No
2.1. Date of the last OEHSA completed	**Date:**
3. Is there information on previous uses of the site/installation that may impact human health?	☐ Yes ☐ No
4. Is there information on off-site sources that may impact human health (natural or man-made)?	☐ Yes ☐ No
5. Is there evidence of human or animal death or disease that may be linked to an environmental exposure?	☐ Yes ☐ No
5.1. If Yes, describe.	
6. Are current or planned worksites, living quarters, and community gathering points appropriately placed to mitigate exposures that could create an acute/immediate health risk (i.e. burn pits, known environmental spills, etc.).?	☐ Yes ☐ No
6.1. If No, provide details.	
7. Does the installation/site have any completed exposure pathways through air with a moderate or higher risk?	☐ Yes ☐ No

8. Is additional air sampling or analysis required?	☐ **Yes** ☐ **No**
9. Are additional air contamination controls required for PAR?	☐ **Yes** ☐ **No**
9.1. If Yes, provide details.	
10. Does the installation/site have any completed exposure pathways through soil with a moderate or higher risk?	☐ **Yes** ☐ **No**
11. Is additional soil sampling or analysis required?	☐ **Yes** ☐ **No**
12. Are additional soil exposure controls required for the PAR?	☐ **Yes** ☐ **No**
12.1. If Yes, provide details.	
13. Does the installation/site have any completed exposure pathways for radiation hazards (ionizing or non-ionizing) with a moderate or higher risk?	☐ **Yes** ☐ **No**
14. Is additional radiation sampling or analysis required?	☐ **Yes** ☐ **No**
15. Are additional radiation controls required for the PAR?	☐ **Yes** ☐ **No**
15.1. If Yes, provide details.	
16. Does the installation/site have any completed exposure pathways for noise hazards with a moderate or higher risk?	☐ **Yes** ☐ **No**
16.1. Is additional sampling or analysis required?	☐ **Yes** ☐ **No**
17. Are there noise controls required for PAR?	☐ **Yes** ☐ **No**
17.1. If Yes, provide details.	
18. Are there Prev Med professionals assigned to the location to relay potential exposure pathways to the Prev Med team with BOS-I?	☐ **Yes** ☐ **No**
18.1. If Yes, LIST names and contact info.	**LIST:**
19. Additional comments:	

3.3. OCCUPATIONAL HEALTH	
1. Have all work centers with occupational hazards been assessed?	☐ **Yes** ☐ **No**
2. How many work centers have been identified?	**Number of work centers:**
2.1. High Risk Processes?	**Number:**
2.2. Moderate Risk Processes?	**Number:**
3. How many exposure assessments are required with very high or high priority?	**Number of exposure assessments:**
3.1. If > 0, provide detail on each.	
4. How many gaps in engineering controls have been identified?	**Number of gaps:**
4.1. If > 0, provide detail on each.	
5. How many gaps in PPE/RP have been identified?	**Number of gaps:**
5.1. If > 0, provide detail on each.	
6. Are members conducting duties that have been identified as hazardous at home station?	☐ **Yes** ☐ **No**
7. Is there work being done that could create an acute health hazard/death?	☐ **Yes** ☐ **No**
7.1. If Yes, LIST (e.g., confined space, using compressed breathing air, industrial X-Ray use, Structural Maintenance, indoor generation use carbon monoxide,	**LIST:**

heat/cold, etc.)	
8. Is HAZMAT purchased from local vendors	☐ **Yes** ☐ **No**
8.1. If Yes, is it being evaluated?	☐ **Yes** ☐ **No**
9. POCs and phone numbers:	
10. Additional comments:	

3.4. MEDICAL CBRN	
1. Are medical personnel coordinating with appropriate personnel (e.g., ATWG, EM, Intel) on CBRN threats?	☐ **Yes** ☐ **No**
2. Does the host nation support the installation CBRN response?	☐ **Yes** ☐ **No**
2.1. If Yes, provide detail on host nation capabilities/gaps.	**LIST:**
3. Are decontamination capabilities available?	☐ **Yes** ☐ **No**
3.1. If yes, select type(s) of decontamination (LIST: Gross, Mobile Unit, Medical Unit, Dry, other)	**LIST:**
4. Does the installation have the appropriate PPE, infrastructure, response plans to protect installation personnel from a CWA incident?	☐ **Yes** ☐ **No**
5. Does the installation have the appropriate detection/response capabilities for existing chemical threats?	☐ **Yes** ☐ **No**
6. Does the installation have the appropriate medical countermeasures for existing chemical threats?	☐ **Yes** ☐ **No**
6.1. If No, to previous 3 questions, provide details on the chemical capability gaps.	
7. Does the installation have appropriate PPE, infrastructure, response plans to protect installation personnel from a BWA incident?	☐ **Yes** ☐ **No**
8. Does the installation have the appropriate detection/response capabilities for existing biological threats?	☐ **Yes** ☐ **No**
9. Does the installation have the appropriate medical countermeasures for existing biological threats?	☐ **Yes** ☐ **No**
9.1. If No, to previous 3 questions, provide details on the biological capability gaps.	
10. Have background levels for radiation been conducted?	☐ **Yes** ☐ **No**
10.1. Type of background radiation assessed.	**Air/Date:** ☐ **Water/Date:** ☐ **Soil/Date:** ☐
10.2. Date last background radiation assessed.	**DATE:**
11. Does the installation have the appropriate PPE, infrastructure, response plans to protect installation personnel from a nuclear/rad incident?	☐ **Yes** ☐ **No**
12. Does the installation have the appropriate detection / response capabilities for existing nuclear/radiological threats?	☐ **Yes** ☐ **No**
13. Does the installation have the appropriate medical countermeasures for existing nuclear/radiological threats?	☐ **Yes** ☐ **No**
13.1. If No to (previous 3 questions), provide details on the nuclear/radiological capability gaps.	
14. POCs and phone numbers:	
15. Additional Comments:	

3.5. TOXIC INDUSTRIAL CHEMICALS/MATERIALS/RADIOLOGICALS (TIC/TIM/TIR)	
1. Has a TIC/TIM assessment been accomplished?	☐ Yes ☐ No
1.1. If Yes, are there TIC/TIMs of concern within a 20-mile radius, including major railways/ highways/waterways?	☐ Yes ☐ No
2. Based on the Risk Assessment, what is the highest level of risk posed from a TIC/TIM? (i.e., Extremely High/High/Moderate/Low)	
3. Are there capability gaps (e.g., response plans, notification procedures, detection, PPE, SIP/evacuation procedures, etc.) that will impact the installation response to a TIC/TIM release posing an extremely high or high risk?	☐ Yes ☐ No
3.1. If yes, provide detail on the capability gap.	
4. POCs and Phone Numbers:	
5. Additional Comments:	

3.6. FERAL ANIMAL AND ENTOMOLOGY	
1. Identify vector-borne pathogens / vectors, vertebrate pests, venomous animals, etc. LIST	
2. LIST any vector-borne and zoonotic diseases.	
3. LIST vectors / pests present.	
4. If on site <48 hours then assess vegetation / terrain for pest harborage, nests, tracks, burrows. Are any of these items present?	☐ Yes ☐ No
4.1. If Yes, what / where?	
5. Are food / water sources present for pests (e.g., standing water from rain, running or large body water, waste refuse area in proximity, human sources of food, etc.)	☐ Yes ☐ No
6. Are there any visual confirmation of pests? Actual sighting of stray / feral animals, snakes, spiders, flies, mosquitoes?	☐ Yes ☐ No
6.1. If Yes, what / where?	
7. Is there evidence of local practices to support feral animal (dog / cat) populations in / around / adjacent to site (i.e., locals feeding animals, local attitude supporting stray animals)?	☐ Yes ☐ No
8. Will any areas change based on seasonality or climate (i.e., monsoon season, hot / cold seasons, decrease in vegetation, etc.)?	☐ Yes ☐ No **Areas:**
9. Are there any flora that are poisonous or that can harbor pests or harm personnel (i.e., poison ivy / oak, cactus, trees where monkeys live, etc.)? Can they be removed?	☐ Yes ☐ No
10. Is the area adjacent to a food / water source for large vertebrates (e.g., a wildlife refuge, large body of water off base, migratory bird flight paths, etc.)?	☐ Yes ☐ No **Animals/Areas:**
10.1 If Yes (previous 3 questions), how can these areas be avoided, removed, isolated, or mitigated?	
11. If on site for >48 hrs. (preferably 72-96 hrs.+), can you perform vector surveillance via sugar/yeast field expedient methods, CDC light trap, BG sentinel, Gravid trap, malaise trap, etc.?	☐ Yes ☐ No
11.1. If yes, LIST insect genera captured, and method used.	
11.2. Are these genera vectors of pathogens or strictly pests (medically significant, pest only, or not significant vector or pest)?	☐ Yes ☐ No

12. Are ticks a concern in this area?	☐ Yes ☐ No
12.1. If Yes, perform vector surveillance, preferably >2x (tick drag, passive attractant (carbon dioxide source) on sticky board/sheet, flagging, etc.)	☐ Completed ☐ N/A
13. Are there standing bodies of water? If yes, perform mosquito larvae dipping. (Bring back larvae/pupa to rear in chamber for ID)	☐ Yes ☐ No
14. Are black flies known to this area?	☐ Yes ☐ No
14.1. If Yes, are there moving bodies of water in proximity; inspect for larvae and collect for ID.	☐ Yes ☐ No ☐ Completed ☐ N/A
15. Is there dense vegetation around site or along perimeter that could harbor large vertebrate pests (think primates, venomous snakes, other wildlife)?	☐ Yes ☐ No
16. Note any other evidence of pest presence that could impact mission and/or personnel.	☐ Yes ☐ No
17. Of any pests identified, can personnel co-exist w/o mitigation?	☐ Yes ☐ No
18. Of any pests identified, what medical mitigation may be required (malaria meds, vax)?	
19. Of any pests identified, which ones require physical mitigation of the environment? LIST	
20. Is there access to or supply of antivenins and anaphylactic kits in the area?	☐ Yes ☐ No
21. POCs and phone numbers:	
22. Additional Comments:	

3.7. EXPOSURE RECORDS	
1. Has the installation / site been added in DOEHRS?	☐ Yes ☐ No
2. Has an OEHSA been accomplished in DOEHRS EH?	☐ Yes ☐ No
3. Have shops, SEGs, personnel, processes, hazards, and controls been established in DOEHRS?	☐ Yes ☐ No
4. Are Prev Med Teams and shop supervisors ensuring rosters in ASIMS and DOEHRS match?	☐ Yes ☐ No
5. POCs and Phone Numbers:	
6. Additional Comments:	

Resources

ISOS (International SOS): https://www.tricare-overseas.com/contact-us /Canada, Central and South America: Phone: 1-877-451-8659 / Europe, Africa, and Middle East: +(44)20-8762-8384/Indo Pacific: +(61)2-9273-2710

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Corporate Website: internationalsos.com

TRANSCOM Patient Movement Requirements Centers (TPMRCs)

- **TPMRC-West:** Commercial: 808-448-1602, DSN: 315-448-1602
- **TPMRC-East:** Commercial: 011-49-6371-478040, DSN: 314-480-8040
- **TPMRC-Americas:** Commercial: 618-817-4200, DSN: 322-817-4200

Attachment 9

AFTER ACTION REPORT TEMPLATE

Unit/Entity

AFTER ACTION REPORT
For
e.g., Op UNIFIED ASSISTANCE (Ebola)

Purpose: AARs serve two primary purposes. First, inform decisions regarding programmed budgeting, strategic/operational/tactical planning, and improvements anywhere in the spectrum of Doctrine Organization Training Materiel Leadership & Education Personnel Facilities-Policy (DOTMLPF-P). Secondly, AAR information may feed into public affairs releases and historian archives. The intent is to summarize key aspects of the operation here and attach detailed analysis, decision points, forces and operational data to retain as an official record. Additionally, subordinate unit AARs may be attached as a matter of record. This template is not prescriptive and directed formats from other commands or formats that better suit a particular mission are acceptable.

SECTION I: Executive Summary.
Include a brief description of the operation/exercise to include scope and purpose, unit(s) involved, dates (be sure to note significant aspects—deployment, redeployment, mobilization, etc.), major locations (country is ok), and a macro roll-up of numbers.

SECTION II: Major Challenges.
Describe the major challenges faced during the event.

SECTION III: Major Accomplishments.
Describe the significant successes that emerged during the event.

SECTION IV: Objective Performance.
Describe where and why there were performance gaps between expected/directed performance and actual performance (good or bad). What were the mitigating or exacerbating factors? Include standardized performance scores if able.

SECTION IVa: *Less than satisfactory performance*
SECTION IVb: *Satisfactory performance*
SECTION IVc: *Better than satisfactory performance*

SECTION V: Major Participants.
Provide a list of the major participants (i.e., units). An exhaustive list is not necessary, but "U.S. Army" is too vague. Identification should be understandable to personnel from other Services.

SECTION VI: Force Disposition.
Include force laydown (who/what and where). E.g. DIRMOBFOR Brig Gen Jones, Camp Snuffy; 321 CRS, Pope AAF. Show enough detail so that future planners and operators can right-size the personnel/equipment deployment packages.

SECTION VII: Lessons Learned.
Summarize successes as well as issues requiring resolution.

Follow Issue (What)-Discussion (So What)-Recommendation (Now What) format for each observation.

Recommendations should include a suggested office of primary responsibility for mitigation/corrective actions.

Note where each observation applies to the DOTMLPF-P spectrum. Note if any observation is a Lesson Learned, and if it has been submitted into the JLLIS.

SECTION VIII: Interoperability.
Provide a narrative of the interoperability issues arising from interaction with:

- *Other airlift/air refueling wings/groups/squadrons*
- *Other USAF major commands*
- *Other US military services*
- *Other US Government agencies (State Department, Department of Homeland Security, etc.)*
- *Host nation*
- *Coalition partners/ allies*
- *Non-governmental organizations (Red Cross, Denton customers, etc.)*

Include enough detail so that others can pre-plan for similar issues in future events.

SECTION IX: By the Numbers.
Summarize number of missions, sorties, short-tons, passengers, fuel offloaded, cost, etc. Add number of personnel deployed (if not indicated in Section VI).

SECTION X: Mission Analysis.
If mission analysis was performed attach slide deck.

SECTION XI: SITREPS.
Insert copies here (SITREPs filed with local HQ and HHQs).

SECTION XII: Orders.
Insert copies of 18 AF orders and HHQ (PLANORD, WARNORD, DEPORD, EXORD, etc.). These orders do not include personnel deployment/TDY orders or crew orders.

SECTION XIII. Video & Audio Content
As applicable, include an appendix with video and/or audio files relevant to any of the observations in SECTION VII.

SECTION XIV: Subordinate Unit AARs.
Insert copies of subordinate unit AARs if available.

SECTION XV: Contact Information.

List of key individuals and their contact information (e-mail, telephone #).

WARNING: Ensure the document is properly marked for overall classification and portion/paragraph markings. See DoDM 5200.01-V2 *DoD Information Security Program: Marking of Information* for specific guidance.

Use only secure systems to transmit classified reports.

The following classification markings are for illustration purposes only.

(U) OPERATION STORM RIDER

(S) Section I: Executive Summary. This operation supported a response to Country X's

Attachment 10

CHECKLISTS

Table A10.1. FFGRL Checklists

PREDEPARTURE
☐ FFGRL team composition and readiness: ☐ All required positions filled or approved substitutions documented ☐ CMRP training currency verified in MRDSS ☐ IMR status "green" verified; immunization group aligned with CR units ☐ Official passports obtained (as needed) ☐ Dual weapons qualification (M-18 and M-4) complete
☐ Deployment timeline, equipment, and processing requirements confirmed with the Medical Readiness Office ☐ Transportation requirements for FFGRL identified
☐ Medical documentation and kits: ☐ DD Form 2766 updated for all personnel ☐ Publications kit built (refer to AFTTP 3-42.24 Attachments 2–4)
☐ FFGR1 UTC and equipment: ☐ Detailed assemblage listing prepared for pack-out and re-supply ☐ MAJCOM/SGX coordination for FFGR1 LOGDET shortfalls ☐ Transportation requirements for FFGR1 identified by AMC/SGX ☐ Hazardous cargo documented; Shipment Declaration of Dangerous Goods forms completed ☐ Iridium SIM activation process confirmed ☐ Deployable laptop with DoD SDC and software suite (HALO, DMLSS, DOEHRS, ILER, JMeWS, TMDS, PMI-ATS, TRAC2ES, DoD Platform One)
☐ Gather medical intelligence data/brief to deploying CRE: ☐ Bed-down considerations reviewed (climate, flood plains, prevailing winds, land usage history) ☐ Endemic diseases, vectors, and common illnesses ☐ Raw water source data and approved bottled water sources verified ☐ Local flora and fauna hazards ☐ Local industrial facilities and toxic industrial hazards ☐ CBRN threat assessment ☐ Existing site surveys pulled and reviewed (DOEHRS, previous teams) ☐ NCMI/TRAVAX and DIA medical intelligence reviewed

☐ Local medical resources identified (hospitals, ER, surgery, ICU, imaging, blood bank) ☐ Medical evacuation procedures identified; distance, response time, emergency numbers, directions
☐ Pre-deployment medical intelligence briefing delivered to deploying commander and all deploying personnel
☐ Communications and information systems: ☐ SIPRNet accounts established ☐ DoD Platform One Impact Level 4 (IL4) account established ☐ Contact data verified (AFMLOC, AMC/SG help desk, TPMRC, ISOS) ☐ Reporting chain confirmed ☐ Contact established with CRE Commander ☐ Reviewed WARNORDs, PLANORDs, EXORDs, DEPORDs, OPORDs
ARRIVAL ACTIONS — INITIAL OPERATIONAL CAPABILITY (IOC) ≤ 24 HOURS AFTER ARRIVAL
☐ Initial site surveillance conducted; input provided to site selection and laydown layout
☐ FFGR1 medical equipment package delivered and accessible to the team
☐ Essential and emergent care capability established
☐ Communication established with the CRE commander; team integrated into Operations Center huddles
☐ Medical tent or facility identified and initial setup started
☐ IFE coverage arranged and plan established; deployed commander briefed
☐ Initial medical briefings delivered (medical tent location, flora/fauna, local hazards, emergency plan)
ARRIVAL ACTIONS — FULL OPERATIONAL CAPABILITY (FOC) ≤ 48 HOURS AFTER ARRIVAL
☐ Medical tent/facility fully established; equipment operational
☐ Patient encounter data is being documented, trends identified and forwarded to the theater surgeon, as needed
☐ Deployment Medical Records (DD Form 2766) have been received and reviewed
☐ Established work shifts
☐ FFGR1 UTC equipment inventoried
☐ Transportation established for patient movement and OEH surveys
☐ Communications and C2: ☐ Satellite/LAN connectivity established ☐ HALO/AHLTA-T, DMLSS, DOEHRS, ILER, JMeWS, TMDS, TRAC2ES operational ☐ SIPRNet access established (if available) ☐ Daily medical SITREP reporting established via DoD Platform One ☐ Links established with theater surgeon, joint/coalition, and civilian healthcare authorities
☐ Local medical capabilities determined

☐ Patient movement: ☐ MEDEVAC (9-line) procedures confirmed ☐ Ground evacuation routes identified; CASEVAC plan and referral procedures established ☐ AE (TPMRC) access confirmed
☐ Safe source of food and potable water established; alternative sources recommended
☐ TIC/TIM and food and water vulnerability assessments initiated
☐ Initial OEH monitoring and surveillance capability initiated; health risk input provided
☐ Mass Casualty plan established and briefed (including casualty collection point)
☐ Aircraft Mishap Response plan established and briefed (be aware of how many aircraft are coming in and out each day and type)
SUSTAINMENT
☐ Daily/routine actions: ☐ Operations Center huddles and applicable working groups attended ☐ Shift-change medical risk brief completed ☐ Medical logistics and resupply coordinated ☐ DNBI tracked and reported ☐ Daily medical SITREP submitted via DoD Platform One ☐ Patient encounters documented ☐ Controlled substances accountability maintained
☐ Preventive Medicine: ☐ Performed environmental reconnaissance of laydown, food facilities, and operating areas ☐ Sanitation oversight and enforcement ☐ Vector surveillance and outbreak investigations completed as needed ☐ Industrial/shop walk-throughs and PPE checks conducted ☐ OEHSA, vulnerability reports, and risk assessments conducted ☐ OEH sampling and food/water surveillance ongoing ☐ Bottled water sources verified in Worldwide Directory ☐ DNBI surveillance system maintained ☐ Thermal stress education and work-rest cycle guidance provided ☐ Communicated health threats and prevention measures to the CRE
☐ Medical Care ☐ IFE coverage and AE access procedures maintained ☐ Built relationships with local/civilian medical counterparts ☐ Local medical capabilities reassessed; deployed commander updated ☐ Human factors monitored (e.g. fatigue, heat/cold stress) ☐ FS serves as OPR for medical site assessment, laydown plan, MEDRED-C, and SITREP
Redeployment / Post-Deployment
☐ Redeployment means verified
☐ Data transition to follow-on medical forces (PAM Team or similar)
☐ Final medical SITREP submitted via DoD Platform One

☐ FFGR1 inventoried and re-packed IAW pack-out list: ☐ Hazardous items documented ☐ Personal items kept out of FFGR1 asset containers ☐ Classified materials packaged, marked, and safeguarded; cleared couriers identified
☐ DOEHRS records updated; data uploaded once connectivity restored
☐ Deployment Medical Records (DD Form 2766) returned to CRE Commander
☐ After Action Report submitted